



Voluntās

WFP's Integrated Cross-Cutting Context Analysis and Risk Assessment in Ukraine

APRIL 2026

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ACRONYMS

AAP	Accountability to Affected People
ADRA	Adventist Development and Relief Agency
AIDS	Acquired Immunodeficiency Syndrome
ART	Antiretroviral Therapy
CCCM	Camp Coordination and Camp Management
CSO	Civil Society Organization
FAO	Food and Agriculture Organization
FGD	Focus Group Discussion
FSL	Food Security and Livelihoods
GAU	Gay Alliance Ukraine
GBV	Gender-Based Violence
HCT	Humanitarian Country Team
HIV	Human Immunodeficiency Virus
IASC	Inter-Agency Standing Committee
ICRC	International Committee of the Red Cross
IRC	International Rescue Committee
IFES	International Foundation for Electoral Systems
IOM	International Organization for Migration
KII	Key Informant Interview
LGBTQIA+	Lesbian, Gay, Bisexual, Transgender, Queer, Intersex, Asexual, and others
MHPSS	Mental Health and Psychosocial Support
MSM	Men who Have Sex with Men
NATO	North Atlantic Treaty Organization
NAPD	National Assembly for Persons with Disabilities
NGO	Non-Governmental Organization
NRC	Norwegian Refugee Council
OCHA	UN Office for the Coordination of Humanitarian Affairs
OECD DAC	Organization for Economic Co-operation and Development's Development Assistance Committee
PLHIV	People Living with HIV
PTSD	Post-Traumatic Stress Disorder
PWID	People who Inject Drugs
SESU	State Emergency Service of Ukraine
TB	Tuberculosis

TCR	Territorial Centres of Recruitment
UAH	Ukrainian Hryvnia
UNAIDS	Joint United Nations Programme on HIV/AIDS
UNDP	United Nations Development Programme
UNFPA	United Nations Population Fund
UNHCR	United Nations High Commissioner for Refugees
UNICEF	United Nations International Children's Emergency Fund
USD	United States Dollar
WASH	Water, Sanitation and Hygiene
WFP	World Food Programme
WHO	World Health Organization



Executive Summary

EXECUTIVE SUMMARY

Background

As Ukraine enters the fifth year of the full-scale invasion, the country remains heavily affected by ongoing hostilities. As of early 2025, 3.7 million people remain internally displaced, 6.9 million are refugees abroad, and over 40,838 civilian casualties have been recorded.¹ The impact is most severe in frontline oblasts such as Donetsk, Kharkiv, Kherson, Luhansk, Mykolaiv, Odesa, Sumy, and Zaporizhzhia.² Continued displacement has deepened poverty, strained social protection systems, and disrupted livelihoods; particularly in rural and conflict-affected areas, where unemployment remains high.³ Social transfers, including pensions and targeted assistance to displaced people, have played a crucial role in preventing further hardship, but coverage may not be reaching hard-to-reach groups such as people without documentation, and hidden groups like Roma communities, LGBTQIA+ individuals, and people living with HIV/AIDS, or men avoiding military conscription.⁴ The psychological toll of the prolonged conflict is also growing, with 63 percent of households reporting mental health challenges related to ongoing uncertainty and displacement.⁵ According to Ukraine's 2025 Humanitarian Needs and Response Plan, 12.7 million people in Ukraine are in need of assistance. Among them, 45 percent are women, 30 percent are older people (60+ years old), 15 percent are children, and 14 percent have disabilities.⁶

Within this context, the humanitarian aid landscape is shifting due to funding constraints and a gradual transition from emergency relief to resilience-building efforts.⁷ Emergency assistance is increasingly concentrated in frontline and war-affected oblasts, while support in central and western Ukraine is being scaled back as international organizations shift their strategies away from short-term emergency aid toward resilience- and development-oriented programming in areas perceived as more stable.⁸ However, humanitarian actors have raised

1 UNHCR. 2024. Multi-year strategy 2025-2027.; UN Human Rights. 2024. Ukraine: Protection of civilians in armed conflict.

2 UNHCR. 2024. *Multi-year strategy 2025-2027*.

3 IOM. 2024. Employment, Mobility and Labour Market Dynamics in Ukraine.; World Bank. 2024. Listening to citizens of Ukraine: the poverty and human impacts of Russia's invasion of Ukraine.

4 World Bank. 2024. *Listening to citizens of Ukraine: the poverty and human impacts of Russia's invasion of Ukraine*.

5 OCHA. 2025. Ukraine Humanitarian Needs and Response Plan 2025. <https://reliefweb.int/report/ukraine/ukraine-humanitarian-needs-and-response-plan-2025-january-2025-enuk>

6 OCHA. 2025. *Ukraine Humanitarian Needs and Response Plan 2025*.

7 OCHA. 2023. *Ukraine Humanitarian Needs and Response Plan 2024*.; SwissInfo. 2024. Donor fatigue looms over humanitarian aid in Ukraine after two years of war.

8 KII 15, INGO staff; KII 18, IO staff; KII 12, IO st

concerns that this shift may create gaps in assistance for vulnerable populations who continue to depend on support in these more stable areas where aid is being scaled back.

As operations become more localized, humanitarian actors have also expressed concerns about the capacity of Ukraine's social security system to take over responsibilities currently handled by international organizations. This is largely due to budget pressures, a shortage of qualified personnel, particularly in social services, and the destruction of essential facilities caused by missile strikes.⁹ Concerns have also been raised about the long-term sustainability of aid delivery, particularly as the war drags on and humanitarian needs continue to grow.¹⁰

Local organizations also worry that cross-cutting aspects of humanitarian work – such as gender equality, accountability to affected people, the prevention of sexual exploitation and abuse, and disability inclusion – may be deprioritized amid international funding cuts, due to limited capacity and competing government priorities.¹¹

As Ukraine navigates these ongoing challenges, a balanced approach between emergency response and long-term resilience-building is essential to ensure that basic needs are met, social tensions between recipients and non-recipients of aid are minimized, and economic recovery is supported.

Study Objective and Method

To support more effective and principled humanitarian action in Ukraine, the World Food Programme (WFP) contracted Voluntas and Info Sapiens to apply the Integrated Cross-Cutting Context Analysis and Risk Assessment (I-CARA). The analysis aimed to deepen understanding of the complex and evolving context and provide inclusive, risk-informed, and contextually grounded humanitarian action in Ukraine.

To implement this assessment, Voluntas and Info Sapiens conducted 20 key informant interviews (KIIs) with representatives of local NGOs, civil society organizations, international organizations, cluster representatives, and local government officials. Additionally, 40 focus group discussions (FGDs) were held with community members, including adult and older women and men, as well as hard-to-reach groups such as internally displaced persons, persons with disabilities, Roma communities, LGBTQIA+ individuals, and people living with HIV/AIDS. To complement and inform data collection, an extensive desk review was

9 Kyiv School of Economics. 2024. *Report on damages to infrastructure from the destruction caused by Russia's military aggression against Ukraine.*

10 SwissInfo. 2024. *Donor fatigue looms over humanitarian aid in Ukraine after two years of war.*

11 KII 20, working group staff

conducted using publicly available sources and materials shared by participants. The data collected was then triangulated and synthesized, with key findings summarized below and detailed throughout the report.

Key findings

Cross-cutting community findings

The war in Ukraine has disrupted access to food and essential services, including healthcare, utilities, water, housing, and education, for both displaced and host communities. Access to these basic services is increasingly undermined by infrastructure damage, rising costs, and displacement.

The war has placed the Ukrainian **healthcare** system under pressure, particularly in smaller towns and frontline areas. Long waiting times, a shortage of specialists, and the high cost of private services restrict access for many, especially for older adults, people with disabilities, LGBTQIA+ individuals, and those living with chronic health conditions such as HIV. Displaced individuals face bureaucratic hurdles, including the need to register with family doctors to access care. Many doctors are at full or limited capacity, forcing people to seek treatment at other, often distant, facilities. In areas like Donetsk and Kherson, limited services are temporarily sustained by mobile clinics and intermittent humanitarian interventions.

As for **utilities and energy**, power outages are frequent, especially in conflict affected zones, disrupting heating, lighting, and cooking. High electricity prices, along with damaged power plants and heating infrastructure, have forced communities to turn to alternative heating sources such as gas, wood, and briquettes – supplies that are increasingly expensive or scarce. These challenges are further compounded by lower pensions and wage growth relative to food price increase, which have not kept pace with utility costs, forcing families to make difficult choices between essential need

Water access and quality also vary significantly. In regions like Vinnytsia, reliance on poor-quality well water is common, while in Donetsk, water output has decreased dramatically, leading residents to depend on irregular water deliveries. Despite significant national increases in water tariffs, access to safe, reliable, and affordable water remains elusive, especially in rural and frontline communities.

While **food** is generally available, it has become less affordable, particularly for female-headed households, the Roma community, and internally displaced people. To cope with food insecurity, individuals resort to cheaper, less nutritious food or skip meals to feed their children. Households in frontline regions face compounded risks due to disrupted supply chains and limited access to fresh produce and protein-rich foods. Although food assistance is provided, concerns about quality, spoilage, and the lack of tailored items for individuals with dietary

restrictions – such as those with diabetes or heart conditions – persist.

Education has been disrupted by destroyed school infrastructure, poor internet connectivity, and persistent power outages. Many children in frontline oblasts have not attended in-person classes since 2022, raising long-term concerns about learning loss. Remote learning has proven especially ineffective for children with disabilities, who require specialized support that is largely unavailable in online formats. Though efforts such as school meals support and winterization initiatives offer some relief, systemic barriers remain.

To address basic needs, cash assistance is widely preferred due to its flexibility, allowing households to prioritize essential expenses such as food, rent, and medicine. Nonetheless, in-kind aid remains critical in frontline and rural areas where access to markets is limited. However, **physical and informational barriers hinder aid access**. Long curfews, reduced public transportation, destroyed infrastructure, and limited digital literacy make it difficult for many especially the elderly, people with disabilities, and rural residents – to reach aid distribution points or complete online applications. Documentation requirements further exclude displaced people and marginalized groups, such as the Roma community, who often lack proper identification

Social cohesion has fluctuated. While tensions initially rose between host communities and displaced individuals, shared hardship and mutual aid have gradually rebuilt trust. In many communities, informal networks continue to provide critical support, such as helping neighbours with daily tasks or sharing food and essentials. Still, perceived inequities in aid distribution remain significant conflict sensitivity risks, threatening community trust and potentially fuelling resentment. These concerns are particularly acute in contexts where aid is viewed as politicized or unevenly distributed, such as when local authorities or influential individuals are perceived to favour certain groups or regions over others. In such cases, community members may believe that access to assistance depends more on personal connections or political affiliations than on actual need

Humanitarian communication and accountability mechanisms are mixed. Aid information is shared via digital platforms like Telegram, Facebook, and Viber, as well as traditional media such as radio, billboards, and SMS. While these efforts are essential, many respondents report confusion and frustration over unclear eligibility criteria and changing requirements. Direct phone calls, hotlines, and face-to-face communication remain crucial for improving clarity and trust – particularly among those with limited digital access or literacy.

Despite efforts to improve **inclusion and feedback**, many data collection respondents reported feeling excluded from decision-making processes related to aid. There is a strong desire for greater transparency, participatory approaches,

and dignity in assistance – such as enabling people to choose needed items or reducing the public visibility of aid collection.

Community-specific findings

Women

Women are increasingly becoming the primary breadwinners in households and taking on roles traditionally dominated by men. However, financial constraints make it difficult to afford basic necessities such as healthcare, food, utilities, and hygiene products, forcing some women to prioritize certain needs over others. Additionally, limited childcare options make it difficult for mothers to balance paid work and caregiving, creating a significant barrier to employment. Amid rising prices, many women continue to engage in informal or part-time work – such as housekeeping, sewing, baking, or recycling materials – due to limited employment opportunities, caregiving responsibilities, or skill constraints. This type of employment, which is often low-paid and lacks social protections, reinforces the existing wage gaps between men and women. These shifts in household roles and economic pressures can exacerbate existing tensions or introduce new risks of gender-based violence (GBV) or exploitation.

Men

Conscription has become a major obstacle for men in Ukraine, discouraging them from public activities like registering for assistance or seeking formal employment. Official jobs require documentation from Territorial Centres of Recruitment (TCR), but fear of mobilization deters many men from visiting TCR offices. Employers are equally hesitant to hire men, anticipating potential changes to the regulations that could lead to conscription. Consequently, many men opt for unofficial work which provides income but lacks stability, legal safeguards, and social benefits. This persistent threat of mobilization heightens their sense of insecurity, restricting movement and everyday routines, and in some cases, compelling men to hide to avoid being drafted.

Older People

Most older data collection participants depend on modest pensions, which can barely cover rising healthcare, food, hygiene, and utility costs. They face isolation, limited mobility, and low digital literacy, which hinders both access to basic services and the ability to receive assistance. Local communities often help fill these gaps by sharing information and supporting older people in registering for and accessing assistance. Additionally, some older people try to supplement their income through work, though chronic health issues and limited physical capacity often prevent them from taking on physically demanding roles. However, even in less strenuous jobs, employers are often reluctant to hire older individuals.

Roma Community

Roma participants reported persistent ethnic discrimination severely restricting their access to public services and leading many to avoid state institutions. This often results in individuals lacking essential identification documents, such as birth certificates or passports, further limiting their access to basic services and assistance. Some NGOs also exclude Roma from aid distribution lists, influenced by stereotypes that depict Roma communities as too isolated to engage with. Additionally, low literacy levels and limited formal education sharply reduce employment opportunities and access to skilled labour markets. Within Roma communities, leadership is generally centralized around a community leader, while the patriarchal cultural and gender norms often grant men greater freedom of socialization, with women expected to limit their interactions to close family circles.

People with Disabilities

Since the escalation of the war, the number of people with disabilities has increased. Individuals with limited mobility face significant challenges accessing water during supply outages, while power cuts leave homebound individuals stranded, and those reliant on electric wheelchairs immobilized due to charging issues. Many facilities, including shelters, remain physically inaccessible due to the absence of elevators or ramps. Additionally, employment rates for people with disabilities remain low, driven by persistent discrimination, stereotypes questioning their ability to perform job duties or maintain long-term employment, and a lack of workplace adaptations. Although this group continues to benefit from aid, assistance has notably declined for those with third-group disabilities because support now prioritizes people in the most vulnerable situations, reflecting stricter eligibility criteria and reduced resources.

Internally Displaced People

Housing remains one of the most urgent issues for internally displaced people, especially in urban areas where high rent costs, limited affordable housing, discrimination by landlords, and unstable rental markets contribute to housing insecurity. Additionally, internally displaced people face challenges securing stable employment due to widespread business closures and relocations caused by ongoing war. However, some displaced people have found opportunities through relocated enterprises or by starting their own business. Others highlighted the need for vocational training programs that provide skills in high-demand fields, such as the IT sector and truck driving. While differences in aid distribution have sometimes caused tension between internally displaced people and host communities – especially when locals perceive that displaced individuals receive more support – aid has also served as a bridge for solidarity. In some cases, internally displaced people have shared food or resources with struggling neighbours, creating informal support networks.

LGBTQIA+

Before the full-scale invasion, LGBTQIA+ individuals in Ukraine were already among the most marginalized, facing widespread stigma and discrimination. The ongoing conflict has dramatically intensified these vulnerabilities, particularly for the estimated 300,000 LGBTQIA+ people who are internally displaced. Many avoid public shelters due to fears of being outed or harassed. Access to gender-affirming care, especially for transgender people, is limited by stigma and administrative obstacles, such as difficulties in receiving the F64.0 diagnosis needed for legal gender recognition. Though public support for LGBTQIA+ rights has grown, targeted support remains scarce. Community members often rely on LGBTQIA+-led organizations, which provide critical services like safe shelters, food assistance, and psychosocial support. Many LGBTQIA+ people avoid non-specialised humanitarian services due to safety concerns, privacy risks, and fear of identity exposure. For this community, inclusive, anonymous, and sensitively delivered assistance is a necessity.

People living with HIV/AIDS

The conflict has disrupted HIV/AIDS care across Ukraine. While access to antiretroviral therapy has resumed in many areas, a shortage of specialists, destroyed infrastructure, and displacement have left many people living with HIV (PLHIV) vulnerable. Confidentiality breaches, stigma, and long wait times further complicate access to care. Most PLHIV prefer to receive aid from specialized NGOs they already trust. Many avoid state-run programs for fear of their HIV status being exposed. Employment remains difficult due to discrimination, and legal protections are weak.

Conclusions

As Ukraine enters the fifth year of full-scale war, the humanitarian landscape remains complex and rapidly evolving. The I-CARA assessment offers a comprehensive understanding of the diverse needs, risks, and coping strategies of affected populations, particularly those in hard-to-reach and marginalized communities. These insights are intended to guide inclusive, context-sensitive humanitarian programming that balances immediate relief with long-term resilience, while also strengthening trust, accountability, and community cohesion.



Access to Aid and Services

Access to humanitarian assistance remains a pressing challenge, especially in frontline and rural areas where infrastructure damage, curfews, and limited transport restrict mobility. While cash assistance is widely preferred for its flexibility, in-kind aid remains vital where markets are disrupted or unavailable. Barriers such as digital exclusion and documentation requirements disproportionately affect older people, persons with disabilities, and Roma communities. Some rely heavily on informal networks or are forced to discontinue medical treatment due to unaffordable costs. Maintaining both cash and in-kind modalities is therefore critical to ensuring inclusive coverage and avoiding further exclusion.



Accountability and Communication

Despite diverse communication efforts, many community members report confusion about aid eligibility and procedures. Direct, personalized communication, via hotlines, phone calls, and face-to-face interactions, remains the most trusted and accessible, particularly in rural and frontline areas. Feedback mechanisms are in place but vary in effectiveness, with participants strongly calling for more transparent, responsive, and dignified approaches to aid delivery that reduce stigma and strengthen community participation.



Inclusive and Conflict-Sensitive Targeting

Perceptions of fairness in aid distribution are closely tied to transparency and participation. Marginalized groups, including Roma, LGBTQIA+ individuals, and people with disabilities, often feel excluded due to rigid documentation requirements and reliance on digital systems. Participants highlighted the importance of flexible, inclusive targeting approaches that recognize diverse vulnerabilities, such as undocumented individuals, older persons experiencing age discrimination, and Roma individuals facing ethnic profiling, to reduce tensions and ensure equitable access.



Protection and Safeguarding

Safety and dignity in aid delivery are paramount, particularly in conflict affected regions where overcrowding, long queues, and exposure to hostilities pose risks. Certain groups, such as LGBTQIA+ individuals, Roma women, and people living with HIV/AIDS, face compounded protection risks due to stigma or fear of discrimination. Tailored and discreet delivery methods, alongside culturally sensitive approaches, are critical to safeguarding dignity and minimizing harm.



Livelihoods and Economic Recovery

The war has severely disrupted livelihoods, with widespread unemployment and reliance on informal work, remittances, and humanitarian aid. Structural barriers—including limited childcare, age discrimination, and accessibility gaps—disproportionately affect women, older adults, and persons with disabilities. Vocational training and reskilling programs, especially when designed with or led by marginalized communities, were identified as valuable pathways to recovery and reducing long-term dependency on aid.



Social Cohesion and Integration

While solidarity has grown in many communities, perceived inequities in aid distribution and persistent cultural or ethnic discrimination continue to strain relationships. Community-led initiatives, shared activities, and inclusive engagement strategies have proven effective in fostering resilience, building trust, and supporting the integration of displaced persons into host communities.



Evolving Gender and Social Norms

The conflict has reshaped gender roles, with women increasingly assuming household provider roles and men facing mobility restrictions and economic marginalization due to conscription. These shifts highlight the need for programming that recognizes changing dynamics, addresses emerging vulnerabilities, and promotes equitable access to services and opportunities for both women and men.



Localization and Coordination

As humanitarian efforts shift from emergency relief toward resilience-building, the role of national and local actors has become increasingly prominent. While local organizations and authorities are well-placed to identify needs, their influence in strategic decision-making remains limited. Strengthening partnerships between international and local actors, while upholding cross-cutting priorities such as inclusion, safeguarding, and accountability, will be essential to ensuring sustainable and context-sensitive humanitarian action.

A young child, wearing a dark knit beanie with a pom-pom, a puffy winter jacket, and patterned pants, is riding a tricycle. The child is smiling and looking towards the camera. The background is a bright, overcast sky. The entire image has a blue tint, and the word "Introduction" is written in large, white, sans-serif font across the middle.

Introduction

INTRODUCTION

Research purpose & scope

To support more effective and principled humanitarian action in Ukraine, the Integrated Cross-Cutting Context Analysis and Risk Assessment (I-CARA) was conducted to deepen organizations' understanding of the complex and evolving context. This approach integrates protection, accountability to affected people (AAP), gender, and conflict sensitivity to identify and mitigate risks, ensuring that strategies and interventions by humanitarian actors are responsive, inclusive, and grounded in local realities.

Voluntas and Info Sapiens (IS) assisted the World Food Programme (WFP) in applying the I-CARA framework, providing comprehensive insights into the Ukrainian context. The analysis supports the design of inclusive, risk informed and context-sensitive humanitarian activities that address the diverse needs of women, men, people with disabilities, and other people in vulnerable situations. Recognizing that conflict and food insecurity affect individuals differently, the exercise also informs targeting strategies.

The analysis identifies potential risks the different groups face, considering threats, vulnerabilities, capacities, and coping mechanisms. It examines differing needs based on gender, age, disability status, and other identity factors to ensure that assistance reaches those most at risk. Additionally, it examines human rights violations, including protection risks, while assessing negative coping strategies related to food security and livelihoods. The study also examines exclusion risks linked to gender, language, and social/health status.

Background

Ukrainian context

Russia's full-scale invasion on February 24, 2022, led to mass displacement, severe infrastructure damage, and significant civilian casualties. The escalation has been described as the largest conventional military attack on a sovereign state in recent history and Europe's worst refugee crisis since World War II.¹²

Now entering its fifth year, the war in Ukraine continues to cause widespread suffering, loss of life, and destruction, putting millions at risk. Mandatory evacuations from front-line communities persist, and humanitarian needs continue to grow.¹³ In August and September 2024 alone, an additional 100,000

12 ARC. 2024. *Ukraine Humanitarian Crisis: Two-Year Report*.

13 IRC. 2024. *Multisectoral Need Assessment Report*.

people were newly displaced.¹⁴ As of early 2025, 3.7 million people remain internally displaced, with an additional 6.9 million refugees abroad.¹⁵

Continued displacement has deepened poverty risks, strained social support systems, and worsened economic instability. Job losses, especially in conflict affected regions, have further compounded the crisis, with more than a fifth of pre-war adult workers losing employment. Unemployment is especially acute in rural areas, where limited economic opportunities have been a persistent challenge. According to a survey of 40,000 respondents conducted by IOM, 39 percent of people in rural areas report a lack of job opportunities, compared to 28 percent in major cities.¹⁶ By 2023, an estimated 1.8 million additional people had fallen into poverty compared to 2020.¹⁷ Without government support, including pensions and social assistance for internally displaced people, nearly three million more Ukrainians would have been pushed into poverty. These social transfers have been a critical lifeline, helping to mitigate the economic impact of the war in Ukraine on the most vulnerable populations.¹⁸ In parallel, the psychological toll of prolonged war is growing, with 63 percent of households reporting mental health challenges due to ongoing uncertainty, violence, and forced displacement.¹⁹

Attacks by Russian Federation across the country have damaged homes, community buildings, and critical infrastructure, leaving residents dependent on external aid. Damage is largely concentrated in the frontline oblasts of Donetsk, Kharkiv, Kherson, Luhansk, Mykolaiv, Odesa, Sumy, and Zaporizhzhia, as well as in areas reclaimed by the Ukrainian Armed Forces.²⁰ Kyiv oblast has also experienced damage from airstrikes and falling debris from destroyed drones.²¹ While Ukraine has reclaimed some of the territory initially occupied, as of February 2025, Russian Federation still controls 20 percent of the country's land. Airstrikes, landmines, explosive remnants of war, and ongoing fighting have resulted in over 40,838 civilian casualties since the full-scale invasion began.²² In response, Ukraine has continued retaliatory drone strikes on Russian military and infrastructure targets.

Intensified attacks – particularly on infrastructure – are causing widespread

14 UNICEF. 2024. *Ukraine Humanitarian Situation Report No. 47*.

15 UNHCR. 2024. *Multi-year strategy 2025-2027*.

16 IOM. 2024. *Employment, Mobility and Labour Market Dynamics in Ukraine*.

17 World Bank. 2024. *Listening to citizens of Ukraine: the poverty and human impacts of Russia's invasion of Ukraine*.

18 World Bank. 2024. *Listening to citizens of Ukraine: the poverty and human impacts of Russia's invasion of Ukraine*.

19 OCHA. 2025. *Ukraine Humanitarian Needs and Response Plan 2025*.

20 UNHCR. 2024. *Multi-year strategy 2025-2027*.

21 Kyiv School of Economics. 2025. *Report on Damages to Infrastructure from the Destruction Caused by Russia's Military Aggression Against Ukraine as of November 2024*.

22 UN Human Rights. 2024. *Ukraine: Protection of civilians in armed conflict*.

damage, especially to the energy sector.²³ The cost of direct damage to buildings and infrastructure is estimated to be USD 176 billion, while reconstruction and recovery needs are estimated at USD 524 – approximately 2.8 times Ukraine's estimated nominal GDP for 2024.²⁴ More than two million homes were damaged or destroyed between February 2022 and February 2024, with new damage occurring daily.²⁵

Targeted attacks on infrastructure have made repairs increasingly dangerous, and workforce shortages due to conscription have delayed critical restoration efforts. Strikes on energy infrastructure have drastically reduced the country's power-generating capacity, disrupting essential services such as water, gas, and heating, impacting homes, shelters, schools, and health facilities.²⁶

The complexity of the situation is compounded by the recent decision of the United States' 2025 to freeze and/or terminate funding for US foreign aid programmes, which will have immediate consequences on humanitarian aid. Since January 2022, Ukraine has received approximately USD 407 billion in international military, financial and humanitarian aid – more than USD 118 billion of which has come from the United States.²⁷ Of these funds, USAID provided USD 2.6 billion in humanitarian aid, USD 5 billion in development assistance, and facilitated more than USD 30 billion in direct US budgetary support for Ukraine.²⁸ The suspension of funding has disrupted numerous ongoing USAID-funded development projects, including critical infrastructure reconstruction, power generation procurement, and the modernization of railway lines and border crossings with the EU. USAID fund cuts will also affect the agricultural sector, as well as NGOs working to combat Russian disinformation and monitor human rights and rule of law standards.²⁹

Ukrainian Institutional Humanitarian Response

At the national level, following the full-scale invasion, several ministries play a key role in humanitarian, development, and rehabilitation efforts in Ukraine. These include the Ministry of Reintegration of Temporarily Occupied Territories, the State Emergency Service of Ukraine (SESU), the Ministry of Social Policy, the Ministry of Health, and the Ministry of Education and Science. These ministries collaborate with international organizations, UN agencies, and donors to coordinate response efforts, ensuring aid reaches those most in need.

23 UNHCR. 2024. *Multi-year strategy 2025-2027*.

24 World Bank. 2024. *Ukraine - Fourth Rapid Damage and Needs Assessment (RDNA4): February 2022 - December 2024*

25 UNHCR. 2024. *Multi-year strategy 2025-2027*.

26 OCHA. 2025. *Ukraine Humanitarian Needs and Response Plan 2025*.

27 Kiel Institute. 2025. *Ukraine Support Tracker*.

28 OSW. 2025. *The US suspends foreign aid: consequences for Ukraine*.

29 OSW. 2025. *The US suspends foreign aid: consequences for Ukraine*.

The Ministry of Reintegration of Temporarily Occupied Territories³⁰ leads IDP assistance, humanitarian access, and reintegration policies, supporting both frontline communities and IDP integration into host areas. SESU is responsible for emergency response, evacuations, and disaster relief, ensuring rapid intervention in crisis situations.³¹ The Ministry of Social Policy manages social assistance programs, including cash-based aid for people in vulnerable situations, while the Ministry of Health oversees medical care, medicines, vaccinations, and the implementation of targeted policies such as those focused on combating Tuberculosis and HIV/AIDs. The Ministry of Education and Science ensures continuity of learning for displaced children through emergency education programs.

Ukraine's decentralized governance system places significant responsibility for humanitarian response at the regional and local levels. Oblast State Administrations (regional governments) coordinate humanitarian efforts within their jurisdictions, working with national ministries and aid organizations to allocate resources. Raion (district) administrations implement regional policies and manage aid distribution. At the local level, Hromadas (municipalities) – including city councils, village councils, and Starostas (village heads) – play an important role in the provision of assistance and management of social services. In rural areas, Starostas help identify needs and liaise with humanitarian organizations to ensure aid reaches people in vulnerable situations effectively. Local authorities also collaborate with international organizations, CSOs, and NGOs to inform targeting strategies and manage logistics for aid distribution.

Further supporting this decentralized system, 1,319 Administrative Service Centres (TSNAPs) operate across the country, providing essential services such as issuing certificates (e.g., IDP certificates), processing permits, and facilitating registration for government assistance programs. In collaboration with UNHCR, multi-service protection centres (MSPCs) have been established. These centres were designed as 'one-stop shops' to help individuals understand their rights, entitlements, and available assistance.³²

Corruption

As Ukraine's humanitarian response relies heavily on decentralized structures and partnerships with local authorities and organizations, ensuring accountability at all levels is critical. However, Ukraine continues to face challenges related to corruption. According to Transparency International's 2023 Corruption Perceptions Index, Ukraine scored 36 out of 100, ranking 104th out of 180 countries, reflecting

30 Ministry of national unity of Ukraine. N.d. *Mission and tasks of the Ministry of Reintegration*.

31 The State Emergency Service of Ukraine. N.d. *Tasks*.

32 Ukraine is Home. N.d. *Multi-service protection centres and TSNAPs*.

gradual improvement but continued vulnerability in governance and public service delivery.³³

A 2024 survey by the Center for International Private Enterprise's Anti-Corruption and Governance Center, which examined corruption in humanitarian aid in Ukraine, found that 13 percent of organizations had encountered bribery in the past two years.³⁴ Bribe requests most frequently came from local government officials, customs authorities, and the police. When problems with local authorities arose, complaints were typically brought to local councils or oblast state administrations; other issues were reported to the police or the Bureau of Economic Security of Ukraine.

Although 84 percent of surveyed organizations said they would be willing to report corruption or fraud, only 72 percent knew where or how to do so. Many chose not to use formal reporting channels, despite those who did generally being satisfied with the outcomes. Instead, it was more common for individuals to share their experiences on social media. This reluctance to report formally was often tied to the lack of clear internal procedures or the poor enforcement of existing ones.³⁵

Some organizations have anti-corruption policies, usually because they were already working with—or hoping to work with—international NGOs, donors, or humanitarian partners that require them. Still, the overall picture points to serious gaps: two-thirds of organizations did not have a standalone integrity or anti-corruption policy, and nearly three-quarters had no dedicated staff focused on preventing or addressing corruption.

Humanitarian context

According to Ukraine's 2025 Humanitarian Needs and Response Plan, 12.7 million people in Ukraine are in need of assistance. Among them, 45 percent are women, 30 percent are older people, 15 percent are children, and 14 percent have disabilities.³⁶

The full-scale invasion had an adverse impact on housing, land, and property issues, leaving many without stable living conditions. Key challenges include damaged or destroyed homes in both occupied and non-occupied areas and restricted access to housing due to ongoing hostilities.³⁷ As such, there is a

33 Transparency International. 2024. *Corruption Perceptions Index: Ukraine*.

34 Center for International Private Enterprise. 2024. *Corruption Risk Assessment: Humanitarian Aid in Ukraine*. (n= 100 humanitarian aid organizations in Ukraine)

35 Center for International Private Enterprise. 2024. *Corruption Risk Assessment: Humanitarian Aid in Ukraine*. (n= 100 humanitarian aid organizations in Ukraine)

36 OCHA. 2025. *Ukraine Humanitarian Needs and Response Plan 2025*.

37 IOM. 2025. *Housing, Land, and Property in Ukraine*.

significant need for legal support and guidance, particularly for displaced individuals and those who have lost their homes or property due to shelling or forced displacement. In addition to securing shelter, many require assistance in navigating government support programs, accessing compensation, and securing legal aid.³⁸

Many people living in frontline oblasts have evacuated to safer areas since the start of the full-scale invasion in 2022, with a mix of intra-oblast and inter-oblast displacement patterns.³⁹ According to 2022 report, 72 percent of internally displaced people relocated to a different oblast from their place of residence, 27 percent remained within their oblast of origin.⁴⁰ The highest inter-oblast displacement movements were recorded from Donetsk to Dnipropetrovsk oblasts (269,000 internally displaced people) and from Donetsk oblast to Kyiv city (117,000 internally displaced people).⁴¹ Certain oblasts, particularly Kherson, Donetsk, and Kharkiv, saw high rates of intra-oblast displacement.⁴² Kharkiv Oblast (366,000 internally displaced people) and Zaporizhzhia Oblast (176,000 internally displaced people) continue to have a significant concentration of displaced people, as many prefer to stay close to their areas of origin.⁴³ Eastern Kharkiv and Dnipropetrovsk oblasts, in particular, have become key hubs for displaced people.⁴⁴ However, increasing Russian Federation incursions are triggering new evacuations, which could push displacement further westward. This trend is particularly concerning for older individuals, who are more likely to choose intra-oblast displacement over relocating to a different region.⁴⁵

Some internally displaced people reside in collective sites set up by Ukrainian authorities in repurposed public buildings such as dormitories, schools, and health facilities. These sites were not designed for long-term habitation and frequently lack essential infrastructure, including accessible water, sanitation, and hygiene (WASH) facilities and functioning bomb shelters. As of mid-2024, approximately 79,000 displaced people with complex needs were residing in nearly 1,800 collective sites across 24 oblasts, representing about 2 percent of all internally displaced people.⁴⁶ Many residents are long-term displaced, with 79 percent having stayed for over 17 months, while an increasing number of sites (46 percent) continue to receive new arrivals, including evacuees. The demand for transit

38 Ibid

39 IOM. 2024. Ukraine internal displacement report, general population survey.

40 Ibid

41 Ibid

42 Ibid

43 Ibid

44 Ibid

45 Ibid

46 OCHA. 2025. Ukraine Humanitarian Needs and Response Plan 2025.

centres has also risen, with over 25,000 evacuees passing through these temporary accommodation hubs in 2024.⁴⁷ Looking ahead to 2025, an estimated 149,000 internally displaced people in collective sites will require Camp Coordination and Camp Management (CCCM) services, which include the provision of essential items, basic repairs, and maintenance to ensure minimum living standards.⁴⁸

Frontline areas

The shifting frontline continues to impact oblasts such as Dnipropetrovsk, Donetsk, Kharkiv, Kherson, Mykolaiv, and Zaporizhzhia.⁴⁹ These areas continue to experience intense hostilities, leaving residents increasingly vulnerable. In 2024, humanitarian needs reached extreme and catastrophic levels, with over 3.3 million people in the east and south requiring assistance.⁵⁰

Security concerns, limited access to social services, housing, land, and property issues, legal assistance needs, and child separation are among the most pressing challenges in the northeast and south where the war remains most intense.⁵¹ Widespread destruction of civilian and critical infrastructure has heightened protection and shelter needs, with many bomb shelters inaccessible to people with disabilities.⁵² In frontline areas, specialized medical services are scarce, and transporting those in need to larger cities, where such services are available, becomes especially difficult during the winter month ⁵³

Security risks near the frontlines significantly complicate aid deliver , as humanitarian workers are exposed to ongoing hostilities, including airstrikes, drone attacks, and targeted violence against aid personnel and facilities.⁵⁴ The destruction of critical infrastructure, such as roads, storage facilities, and power grids, further hampers aid distribution, making it difficult to transport and store supplies.⁵⁵ Additionally, newly introduced permit systems in some frontline oblasts, such as Kherson and Donetsk, add bureaucratic difficulties, delaying assistance t those in urgent need.⁵⁶ With winter exacerbating these challenges through power outages and harsh weather conditions, humanitarian operations remain severely constrained, forcing organizations to rely on local volunteers and authorities for

47 Ibid

48 Ibid

49 Ibid

50 OCHA. 2024. *Ukraine Humanitarian Needs and Response Plan 2024*.

51 United Nations Human Rights. 2025. *The impact of the armed conflict and occupation on children's rights in Ukraine.*; IOM. 2025. *Housing, Land, and Property in Ukraine.*; OCHA. 2025. *Ukraine Humanitarian Needs and Response Plan 2025*.

52 OCHA. 2025. *Ukraine Humanitarian Needs and Response Plan 2025*.

53 WHO. 2024. *Factsheet access to health services in Ukraine frontline oblasts (2023-2024)*.

54 OCHA. 2025. *Ukraine Humanitarian Needs and Response Plan 2025*.

55 Ibid

56 Ibid

last-mile deliveries in high-risk areas.⁵⁷

Individuals who remain near the contact line face compounded challenges, as damaged infrastructure, market disruptions, and financial hardships limit access to food, water, and other essential services.⁵⁸ In areas where markets are non-functional, in-kind aid is provided to ensure access to essential goods. However, as this aid is standardized, it may not fully meet the diverse needs of the affected population.⁵⁹

As frontline conditions deteriorate, many individuals seek relocation. However, without adequate protection, their vulnerability increases. Economic constraints drive some people, including single female-headed households, people with disabilities, and older people, to remain in or return to frontline areas after evacuating, as they cannot afford to live as internally displaced people in safer regions.⁶⁰ According to a survey of returnees to frontline areas, 27 percent resettled in their original settlements or oblasts, while five percent are living in areas experiencing active hostilities.⁶¹ While family reunification and access to employment were the primary reasons for return, many continue to face serious challenges due to economic insecurity and damaged or inadequate infrastructure.⁶² To address this, NGOs are advocating for a stronger humanitarian response in safer oblasts to encourage displaced people to remain there rather than returning to high-risk areas.⁶³

In April 2024, new legislation lowered the mobilization age from 27 to 25, increasing the number of civilians eligible for conscription.⁶⁴ This has intensified recruitment efforts, including raids on public spaces.⁶⁵ While intended to bolster defence efforts, these changes have introduced new protection concerns and stressors among the civilian population. Upon return some soldiers experience post-traumatic stress disorder (PTSD) and other mental health struggles, which, in some cases, have led to incidents of GBV.⁶⁶

57 Ibid

58 OCHA. 2025. *Ukraine Humanitarian Needs and Response Plan 2025*.

59 Ibid

60 IMPACT. 2024. *Unsafe returns: what makes refugees return to Ukraine and settle in the frontline areas of the country?*

61 Ibid

62 Ibid

63 KII 16, cluster staff

64 AP News. 2024. Ukraine Lowers its conscription age to 25 to replenish its beleaguered troops.

65 Euronews. 2024. *Recruiters raid Kyiv venues in search of Ukrainians not registered for conscription*.

66 CARE International. 2023. *Rapid Gender Analysis: Ukraine*

A shift in humanitarian priorities

Since Russian Federation launched its full-scale invasion of Ukraine in February 2022, the humanitarian response has developed into a large-scale, highly coordinated effort that continues to adapt to the evolving needs on the ground. The initial focus was on emergency relief, addressing mass displacement, food insecurity, and urgent healthcare needs.⁶⁷ This phase then rapidly scaled up under the UN's Level 3 emergency designation—its highest classification—which enabled the swift mobilization of international resources and personnel.⁶⁸

In 2023, the response began shifting from emergency relief toward resilience-building, particularly in regions facing long-term displacement and the breakdown of essential services, infrastructure, and local governance systems due to the prolonged impact of the war.⁶⁹ Humanitarian assistance became increasingly concentrated in frontline and war-affected oblasts, focusing on those directly impacted by the war, as well as transit hubs, collective sites, and temporary shelters.⁷⁰ Meanwhile, other regions are shifting toward development-focused interventions, with support from International Organizations (IOs) moving away from direct humanitarian aid toward repairs, legal aid, and psychological support services.⁷¹ In 2025, in response to funding reduction, and in line with the Emergency Relief Coordinator's call for a 'Humanitarian Reset,' and under the guidance of the Humanitarian Coordinator, the humanitarian community in Ukraine has undertaken a principled, evidence-based reprioritization of the 2025 Humanitarian Needs and Response Plan (HNRP). The process was informed by severity data, updated needs assessments, partners presence, and consultations with humanitarian actors. The revised response is now structured around four strategic priorities:

- Supporting the most vulnerable who remain close to the front line
- Evacuations
- Emergency response after strikes
- Humanitarian contributions to the most vulnerable among the IDPs including those in collective centers.

To safeguard life-saving operations, activities not directly contributing to the four core priorities have been deprioritized, particularly in areas of lower severity or

67 OCHA. 2022. *Ukraine – Humanitarian Response Plan 2022*.

68 Inter-Agency Standing Committee. 2022. *System-wide scale up protocols activated for the Ukraine response*.

69 OCHA. 2024. *Ukraine Humanitarian Needs and Response Plan 2024*.

70 KII 15, INGO staff; KII 18, IO staff

71 KII 12, IO staff

where government capacity is stronger. However, these activities remain part of the HNRP and will continue to be monitored⁷².

This evolution in strategy has occurred alongside growing concerns over donor fatigue and anticipated funding gaps, which are compelling humanitarian organizations to scale down their operations. This is prompting a shift toward sustainability strategies, including the delegation of certain services to the Ukrainian government and civil society actors. While these adaptations aim to safeguard core humanitarian functions, they also raise questions about the long-term viability of aid delivery, especially as the war in Ukraine prolongs and needs continue to persist.⁷³

So far, participation and leadership in coordination structures have shown uneven progress. While local and national NGOs increasingly participate in coordination forums, particularly at sub-national levels, Ukrainian actors still have limited opportunities for meaningful leadership and influence over decision-making. This includes limited authority to shape priorities, co-design interventions, or allocating resources. Despite their significant operational contributions – often implementing more than half of program activities – local organizations continue to be seen primarily as implementing partners rather than strategic co-leaders. Structural barriers such as lack of funding for cluster participation, language limitations, and inadequate inclusion in strategic planning continue to hinder the advancement of a truly localized humanitarian response.⁷⁴

Interviews with International Organisations representatives describe this shift as a strategic response to funding constraints. However, it has raised concerns about people in vulnerable situations in western and central Ukraine, where assistance – whether food, non-food items, or cash-based interventions – remains vital.⁷⁵ IO actors have emphasized that the transition away from these regions must be handled carefully to prevent disruptions in support for those still in need.

As humanitarian organizations prepare to scale down operations, cluster representatives have raised concerns that Ukraine's social security system may not be fully prepared to take over.⁷⁶ This is largely due to a shortage of qualified personnel, particularly in social services, and the destruction of essential facilities caused by missile strikes.⁷⁷ These challenges have severely strained service delivery, making it increasingly difficult to provide consistent support to affected

72 OCHA, Ukraine HNRP 2025 (April)

73 SwissInfo. 2024. *Donor fatigue looms over humanitarian aid in Ukraine after two years of war.*

74 Refugees International. 2024. *Annual Ukraine Localization Survey 2024.*

75 KII 17, cluster staff

76 KII 18, IO staff

77 Kyiv School of Economics. 2024. *Report on damages to infrastructure from the destruction caused by Russia's military aggression against Ukraine.*

communities.⁷⁸

Despite these challenges, IOs continue to collaborate with national and local authorities, coordinating efforts and pooling resources to sustain aid delivery wherever possible.⁷⁹ However, local NGOs worry that cross-cutting aspects of humanitarian work – such as gender equality, accountability to affected people, the prevention of sexual exploitation and abuse, and disability inclusion – may be deprioritized due to limited capacity and competing state priorities.⁸⁰

78 KII 13, cluster staff

79 KII 3, local NGO staff

80 KII 20, working group staff

A woman wearing a patterned headscarf and a light-colored apron over a dark shirt is walking away from the camera through a field of tall grass. She is carrying a wooden crate in her right hand and a metal bucket in her left hand. The entire image is overlaid with a semi-transparent blue filter. The word "Methodology" is written in white, sans-serif font across the middle-right portion of the image.

Methodo- logy

METHODOLOGY

Analytical Framework

The analytical framework was designed with the goal of establishing an in-depth understanding of the Ukrainian context by examining key components such as demographics, access to and provision of basic services, economy and livelihoods, community structure, participation in assistance provision, conflict dynamics, socio-cultural issues, protection risks, and accountability of humanitarian actors. This contextual understanding helped identify and evaluate the risks and vulnerabilities faced by different communities and informed the development of mitigation strategies, ensuring that future interventions are both appropriate and inclusive.

{The full analytical framework with indicators measuring each subcomponent is included in Annex A of the report}

Data Collection

To implement the I-CARA, three modes of data collection were employed: 1) desk review, 2) key informant interviews (KII), and 3) focus group discussions (FGD). Data collection was conducted between 8 October and 20 November 2024 and analysed using Atlas.ti and Excel. General scientific research methods were applied, including generalization, analysis and synthesis, and comparison. Throughout the data collection phase, Voluntas and InfoSapience adhered to the principles of do no harm, conflict sensitivity, informed consent and anonymity, and gender sensitivity, ensuring safe and inclusive participation across gender groups.



Desk Review

A comprehensive desk review was conducted to enhance the understanding of the I-CARA framework and the broader Ukrainian context. It focused on key topics such as gender, disability, inclusion, conflict dynamics, and factors driving food security. The review helped shape the analytical framework and supported the triangulation of data collected through the various primary research methods. {For a full list of reviewed documents and sources, see Annex B}



Key Informant Interviews (KIIs)

Voluntas and Info Sapiens conducted 20 KIIs, all held remotely via Zoom or phone. Two KIIs included two participants, bringing the total number of interviewees to 22. Among them were 16 women and 6 men representing local NGOs, UN agencies, and government officials. The interviewees and the groups they represent were identified in consultation with the WFP team, who also provided the contact

information. The KII guides were tailored to each group, though similar themes were explored across all interviews.

Of the KIIs, five were conducted with representatives of local NGOs supporting people with disabilities, Roma communities, LGBTQIA+ individuals, survivors of gender-based violence (GBV), people living with HIV/AIDS, and internally displaced people. Two interviews were conducted with WFP partner NGOs. Ten interviews were conducted with staff from CARE, IMPACT, UNHCR, UNFPA, UNAIDS, WFP, and two Food Security and Livelihoods (FSL) cluster coordinators from FAO and WFP. Lastly, three interviews collected insights from local government officials.

 Focus Group Discussions (FGDs)

Voluntas and Info Sapiens conducted 40 FGDs with key populations, including women, men, older people, internally displaced people, people with disabilities, the Roma community, people living with HIV/AIDS, and members of the LGBTQIA+ community. FGDs were conducted with the different groups to better understand the challenges, needs, and preferences of various population groups. Each FGD included 6 to 7 participants, which allowed for a productive discussion where everyone had space to share their views. In total, 249 individuals participated in the FGDs divided across the groups as follows:

Group	Sample	Group	Sample
Men (18-59)	44	People with disabilities	13
Women (18-59)	49	Roma community	13
Older men (59+)	50	LGBTQIA+	6
Older women (59+)	43	People living with HIV/AIDS	6
IDPs	25		

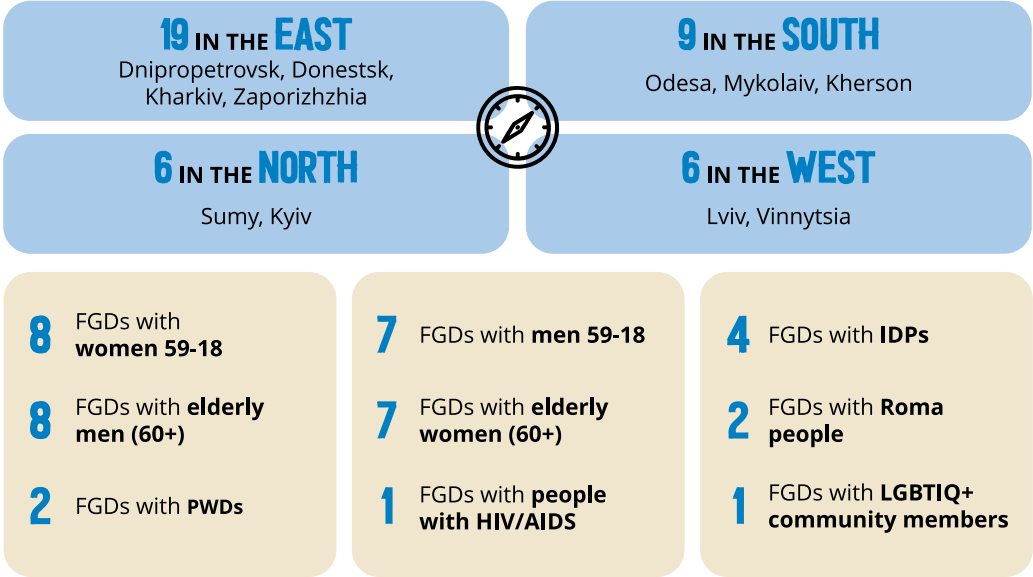
Participants were selected through a combination of approaches. Those from vulnerable groups (e.g. people with disabilities, people living with HIV/AIDS, LGBTQIA+ individuals) were recruited through NGOs that work closely with these communities. Older people, as well as adult men and women, were recruited through Info Sapiens’ local teams. In some cases, WFP representatives supported recruitment by sharing contact information of local activists who were able to assist in reaching participants.

In line with WFP guidelines, Voluntas and Info Sapiens aimed to maintain a balanced mix of participants who had received WFP assistance and those who had not across all FGDs. Overall, 148 participants were recipients of WFP assistance and 101 were not. To reduce the risk of response bias, the consent form used prior to data collection clearly stated that participation in the discussions would not result in any additional assistance nor would it impact any assistance they currently receive. Those who had received WFP assistance were recruited with the

help of local WFP representatives and partners. All participants were screened using a standardized questionnaire to ensure they met the parameters of the group.

The 40 FGDs were held across eleven Oblasts: Dnipropetrovsk, Donetsk, Kharkiv, Kherson, Kyiv, Lviv, Mykolaiv, Odesa, Sumy, Vinnytsia, and Zaporizhzhia, with nine target groups. The figure below shows the number of FGDs carried out in each oblast and the distribution across the different groups. {For a detailed breakdown of FGDs per oblast, please refer to Annex B}

Figure 3: Number of FGDs (per area and target audience)



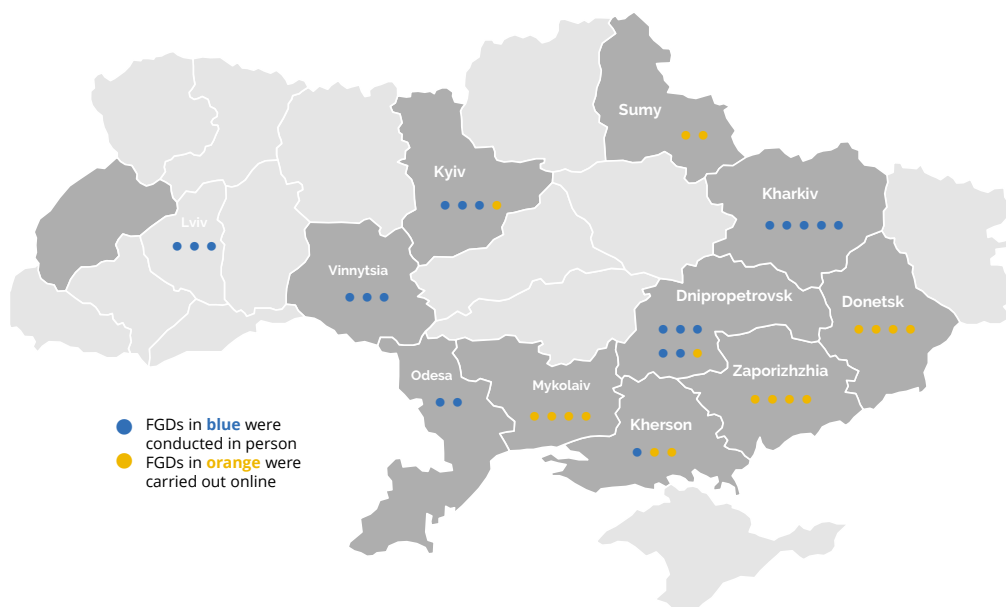
Of the 40 FGDs, 22 were conducted in person and 18 remotely. To maintain flexibility and adaptability, modifications were made in collaboration with WFP during data collection in response to security concerns or sensitivities. Below is an overview of the adjustments made by Oblast:

- **Kharkiv:** : FGDs initially planned as remote sessions were conducted in person, as the situation was deemed sufficiently safe, and frequent power outages made remote sessions challenging.
- **Zaporizhzhia:** FGDs originally planned as in-person sessions were held remotely due to the worsening security situation.
- **Mykolaiv:** The FGD with HIV/AIDS participants was conducted in Mykolaiv, where the strong network and active presence of the local partner, Alliance Global, facilitated participation. The FGD was held remotely for participant safety and confidentiality .
- **Kherson:** FGDs were conducted remotely as planned, except for one session

with the Roma community, which was held in person to better accommodate the need of the target audience.

- **Dnipropetrovsk:** FGDs were conducted in person as planned in Dnipro, with local partners bringing beneficiaries from Nikopol, Kryvyi Rih, and Synelnykove. However, one FGD with the Roma community was moved to a remote format to accommodate their desire for privacy and convenience.
- **Kyiv:** Three FGDs were conducted in person as planned, while one, with the LGBTQIA+ community, was held remotely to accommodate sensitivities, as participants preferred not to meet in person.
- **Sumy, Vinnytsia and Kyiv:** No WFP beneficiaries were invited to the FGDs as local partners reported having no available contacts. Instead, individuals receiving assistance from other organizations, as well as non-beneficiaries, were invited to participate.
- **Dnipropetrovsk and Kherson:** Identifying a sufficient number of Roma community members proved challenging and required additional support from WFP managers and local partners to ensure enough participants were invited.
- **Odesa:** An FGD with women aged 59+ was planned with a fifty-fifty split between WFP and non-WFP beneficiaries. As there were no WFP beneficiaries in this age group in Odesa, the session was conducted with recipients of other aid programs and non-beneficiaries.

Figure 4: Distribution of FGDs (Online/In person)



Challenges and Limitations

Several challenges and limitations affected the scope of this study and the data collected. As such, the following factors should be taken into account when interpreting the findings presented in this report

Scope of qualitative findings

This study used qualitative methods to explore the perspectives of selected participants. While these insights offer depth and contextual understanding, qualitative findings are not intended to be statistically generalizable to the broader population. As such, they reflect the specific experiences of those consulted and should be interpreted as indicative rather than representative. To strengthen the reliability of the analysis, findings were triangulated across multiple sources and respondent groups.

Limited geographic scope

Data collection was conducted in 11 out of 24 oblasts in Ukraine, limiting nationwide representativity and generalizability. These oblasts were selected by WFP based on programmatic priorities. Additionally, only one FGD was conducted per group in each oblast, reducing the diversity of perspectives captured within each oblast, and providing limited opportunities for triangulation.

Limitations of remotely-led FGDs

Due to security concerns, FGDs were conducted remotely in five of the eleven target oblasts. Three of these oblasts are located on the frontline, while the remaining two are subject to ongoing hostilities. While remote FGDs were necessary, challenges related to limited digital literacy were encountered during the FGD with Roma community members. To address this, participants with low digital literacy were brought together in one location, where researchers assisted them in connecting to the remote session

Low representation of hard-to-reach respondents

Unlike men, women, and older people, who participated in FGDs across all oblasts, FGDs with people in vulnerable situations – including internally displaced people, people living with HIV/AIDS, people with disabilities, LGBTQIA+ individuals, and the Roma community – were not conducted in every oblast. As a result, findings related to these groups may not be fully representative of each group's perspectives and experiences.

Inherent research biases

Assessing the needs of people in vulnerable situations can be prone to certain potential cognitive and research biases, which are key to consider in interpreting the findings

- **Social desirability bias.** Respondents may sometimes provide answers they believe are more socially acceptable, which can influence the accuracy of findings. While this was not a widespread issue across most focus group discussions, it was noted during the FGD with Roma participants, where some individuals frequently looked to community leaders or older neighbours for validation while responding.
- **Cognitive dissonance bias.** Respondents may experience discomfort when acknowledging negative aspects of their experience in their community and country. They may downplay or rationalize these aspects and how they perceive them to resolve this discomfort.
- **Confirmation bias.** Given the ongoing conflict and tensions, there was a risk that respondents sought out information that aligns with their existing beliefs or the predominant views of the group they identify with. This can lead to confirmation bias in their responses.

Lessons from Adapting the I-CARA Framework to the Ukrainian context

The I-CARA framework includes a set of qualitative questions designed for implementation across various contexts. To align with the Ukrainian context and the stakeholders involved in data collection, a subset of these questions was selected. These questions were drawn from various question menus, including risk assessment, socio-cultural and gender norms, exclusionary practices, AAP, and stakeholder analysis. To ensure relevance and sensitivity to the local context and target communities, some questions were refined and rephrased.

One key takeaway from this process is that to better assess community-level dynamics, particularly in local communities, general qualitative questions, such as ones assessing access to different basic needs and services, should be adapted into a quantitative format and used in a community-wide survey with a larger sample. This would make the findings more representative while also being a cost-effective approach. A survey would also help address several challenges and limitations outlined earlier: it reduces research bias, eliminates the need for in-person participation, and allows for broader inclusion of people in vulnerable situations. To account for digital literacy issues, surveys could also be conducted by telephone (phone call) or in person.

A survey of this kind could provide a foundation for designing data collection tools, with FGDs then used to add depth and nuance to the findings. This approach was partially implemented in this study by converting the AAP section of the FGD guide into a short telephone survey conducted during participant recruitment, helping to shorten the length of FGDs.

A photograph of a woman smiling and holding a baby, overlaid with a blue gradient. The woman is on the left side of the frame, looking down at the baby. The baby is on the right side of the frame, partially visible. The text "Main Findings" is written in white on the right side of the image.

Main Findings

MAIN FINDINGS

The following sections present findings according to the analytical framework, covering service delivery and assistance, economy and livelihoods, community structure and participation, conflict dynamics, socio-cultural and gender norms, protection risks, and the accountability of humanitarian actors. It begins with an overview of cross-cutting community findings that are common across all groups involved in data collection. This is followed by a detailed analysis of community-specific findings, with dedicated sections for each group, including adult women and men, older people, the Roma community, people with disabilities, internally displaced people, LGBTQIA+, and people living with HIV/AIDs.

Cross-cutting community findings

Food and basic service delivery and assistance

Across all population groups assessed in Ukraine as part of this study, the most pressing needs include healthcare, utilities, financial stability, food security, housing, and education. Access to these services is becoming increasingly strained due to economic instability, ongoing conflict, displacement, and infrastructural damage.⁸¹

Healthcare

Healthcare remains one of the most urgent needs, with long waiting times to see a doctor and a shortage of specialists creating significant barriers to accessing necessary treatment. Private healthcare, often considered an alternative to the state-funded system, is unaffordable for people in vulnerable situations – such as older adults, people with disabilities, and internally displaced people – due to its high cost. These barriers to healthcare access disproportionately affect those with greater and more frequent healthcare needs, including LGBTQIA+ individuals, people living with HIV (PLHIV), older people, and people with disabilities.

Government clinics are overburdened, particularly in smaller towns, forcing many to travel long distances for treatment. In some parts of Donetsk oblast, damaged hospital infrastructure has left mobile clinics as the only option,⁸² while in Mykolaiv oblast, humanitarian organizations like the Red Cross provide occasional free medical services, though coverage remains inconsistent. Bureaucratic obstacles, such as the requirement to register with family doctors, make it more difficult for internally displaced people to access healthcare. As a result, they often have to

81 IOM. 2025. *IOM Aims to Aid Two Million People in Ukraine in 2025 as War and Displacement Continue*.

82 FGD 1, with women (18-59), Donetsk oblast

visit multiple institutions before being admitted for treatment.⁸³

Utilities, Heating, and Water

The cost and availability of utilities, including electricity, heating, and clean water, are major concerns. Frequent power outages disrupt daily life, making it difficult for people to maintain essential heating and cooking systems. Pensions and wages have not kept pace with rising utility costs, forcing many families to prioritize food and utilities over other basic needs. In Dnipropetrovsk oblast, high electricity prices and damaged infrastructure – such as destroyed municipal heating plants, compromised power lines, and non-functional heating plants – have forced residents to turn to alternative heating sources. Although gas remains common, supply disruptions have led many to prioritize wood and briquettes, which they either collect themselves or receive through assistance. In areas where central heating systems are not functional, residents must use solid fuels like firewood.

Access to safe and reliable water varies widely across the different oblasts. In Vinnytsia oblast, residents depend on wells, but the water is often undrinkable due to high sediment levels. In regions affected by active hostilities, tap water shortages can last for days. In Donetsk oblast, daily water output has dropped from 280,000 cubic meters in 2021 to an average of just 120,000 cubic meters. As a result, residents now rely on irregular water deliveries, leaving many without a stable supply.⁸⁴ Despite a 32 percent national increase in water tariffs in June 2023,⁸⁵ some respondents reported that water quality remains poor, forcing many households to purchase bottled water or travel long distances to collect bottled water from water distribution points, this is particularly adds more problems to persons with disabilities or older people, whose access to water could depend on the generosity of others.

Food Security

As reported by FGD participants across the different community groups, while food is available, affordability has increasingly become a challenge. Physical access to food is also a concern, particularly for people living in regions affected by active hostilities. Many households, particularly those headed by women or marginalized groups of people like the Roma community, struggle to afford basic food items. As a result, 59 percent of female-headed households purchase cheaper, often less nutritious, food – compared to 50 percent of male-headed households.⁸⁶ Furthermore, 16 percent of female-headed households reported restricting their

83 FGD 24, with IDPs, Mykolaiv oblast

84 Antikor. 2024. *In the occupied Donetsk, water supplies delivered to the city have halved.*

85 Ukrainian News. 2023. *Water Tariffs Rising in Ukraine: What Will Be the New Prices from July 1?*

86 REACH. 2023. *Multisectoral Needs Assessment (MSNA) – Gender Focus Brief (March 2023).* (n=13,449 households)

own food intake so that small children could eat, compared to 9 percent of male-headed households. This reflects the heightened vulnerability of female-headed households, who are more likely to be sole caregivers of children.⁸⁷

Findings from the latest Joint Food Security and Livelihood Assessment (JFSLA) confirm these trends. The assessment revealed that food insecurity affects between one-fifth and one-third of households in frontline and bordering oblasts, with Donetsk and Zaporizhzhia oblasts among the most affected.⁸⁸ Approximately 38 percent of households surveyed reported food as their most urgent need in the coming six months. Households near the frontlines face severe access constraints due to disrupted supply chains, displacement, and active hostilities, which compound affordability issues and reduce the availability of fresh and protein-rich foods. Meat, along with fruits and vegetables, was particularly out of reach for many families, especially displaced households and those with low incomes.⁸⁹

In response to food insecurity, some families turn to self-production, such as small-scale gardening or raising poultry. This enables them to offset market price fluctuations and, in some cases, earn income by selling surplus produce.⁹⁰ Others reported taking on part-time jobs to supplement their income and ensure that they can afford food alongside other expenses. Some also rely on local barter systems, trading homemade goods for essentials like food and hygiene products.⁹¹

Concerns regarding the quality and appropriateness of food assistance were highlighted by FGD participants. Some reported that poor storage conditions occasionally lead to receiving spoiled goods, while others said they had been given items close to their expiry date. For individuals with specific dietary needs—such as those managing diabetes, heart disease, or other chronic illnesses—receiving generic food assistance that does not align with their dietary needs can sometimes pose serious health risks. Participants emphasized that high-sugar, high-fat, or heavily processed foods sometimes included in aid distributions are unsuitable for people with chronic conditions or dietary restrictions and hence fail to meet their needs. As highlighted by a Zaporizhzhia oblast resident, some aid packages include products such as pork fat, which may be inappropriate for older people or people with health restrictions. Ensuring that food assistance is both safe and appropriate is therefore critical for public health and for effectively supporting those in need.⁹²

87 REACH. 2023. *Multisectoral Needs Assessment (MSNA) – Gender Focus Brief (March 2023)*. (n=13,449 households)

88 WFP, FAO and KSE. 2025. Joint Food Security and Livelihood Assessment of Frontline and Bordering Regions in Ukraine. (sample: 10,419 households)

89 FGD 36, with older men (60+), Lviv oblast

90 WFP, FAO and KSE. 2025. Joint Food Security and Livelihood Assessment of Frontline and Bordering Regions in Ukraine. (sample: 10,419 households)

91 FGD 5, with women (18-59), Kharkiv oblast

92 FGD 19, with older men (60+), Zaporizhzhia oblast

The Joint Food Security and Livelihood Assessment of Frontline and Bordering Regions in Ukraine also found that the household food security level was a concern, especially for households with vulnerable members like persons with disabilities or more than three children, highlighting the need for more tailored assistance to support them.⁹³

Education Barriers

Since the start of the war in Ukraine, access to education has become a major challenge, particularly for children in regions affected by active hostilities. Many schools have been destroyed, forcing students to rely on remote learning, which is ineffective in a area with unstable internet. Frequent power outages further disrupt remote education, making consistent learning impossible for many students.⁹⁴ Children with disabilities are particularly affected by the shift to online education, as they require specialized attention that is difficult to provide remotely. In frontline oblasts, many children have not attended in-person school since 2022, leaving families deeply concerned about the long-term educational impact of the war.⁹⁵

Economy and livelihoods

Despite the war's impact on the economy, some industries and businesses have relocated to safer areas. Sectors such as manufacturing, social services, the food industry, education, trade, and municipal jobs continue to provide employment opportunities.⁹⁶ However, the departure of many men to the front lines while others are trying to evade military service has created significant workforce shortages, which cannot be fully compensated for by the rest of the population.



Our city has now received new trolleybuses. But there is no one to work on them. There are young people who could work, but they are simply hiding. They will not even go there to that trolleybus park or bus, because they will be taken to the recruitment centre.

FGD 23, with older men (60+), Mykolaiv oblast



At the same time, strict formal education requirements for certain roles pose additional barriers to employment, even for individuals with relevant experience.

⁹³ WFP, FAO and KSE. 2025. Joint Food Security and Livelihood Assessment of Frontline and Bordering Regions in Ukraine. (sample: 10,419 households)

⁹⁴ United Nations Ukraine. 2023. *Attacks on Ukraine's Electricity Infrastructure Threaten Key Aspects of Life as Winter Approaches*.

⁹⁵ Save the Children. 2024. *Attacks on Education in Ukraine Double in 2024, Leaving Some Parents Terrified to Send Their Children to School*.

⁹⁶ IOM. 2024. *Employment, Mobility and Labour Market Dynamics in Ukraine*.



For example, in our community, we need an accountant. But we need a person with higher education. I have an accountant's profession, but I graduated from a technical school, and I do not have a higher education.

FGD 26, with women (18-59), Mykolaiv oblast



Job opportunities are largely concentrated in cities, making it difficult for people in rural areas to find employment. This challenge is further compounded by inadequate public transportation and security concerns, which make commuting more difficult.⁹⁷



Even the free bus that was supposed to take people to Sloviansk every Tuesday has been cancelled. It costs one hundred and fifty hryvnias to travel twenty-five kilometres. Some people don't have that kind of money.

FGD 2, with men (18-59), Donetsk oblast



Beyond these limitations, middle-aged people who have not yet reached retirement age face particular obstacles. Employers often hesitate to hire individuals over 50 for skilled positions with stable wages, citing concerns about their health and the relevance of their experience.⁹⁸

When government and humanitarian aid fall short, communities rely on social capital, which includes support from friends, neighbours, or extended family. For some participants, stable remittances – mainly from family members and, in rarer cases, friends abroad help cover urgent expenses like utilities and food; for others, remittances arrive sporadically, if at all. Some reported relying on an array of coping mechanisms, including selling personal belongings and drawing on savings. To build resilience, participants expressed a strong need for capacity building and reskilling opportunities, such as IT training, financial literacy programs, and vocational courses (e.g., sewing, hairdressing). These initiatives not only bolster household income but also foster entrepreneurship – particularly where business development funds or reduced bureaucratic barriers enable small enterprises to flourish.

⁹⁷ FGD 2, with men (18-59), Donetsk oblast

⁹⁸ KII 3, local NGO

Social cohesion

Across much of Ukraine, social cohesion has fluctuated between moments of tension and solidarity, yet overall, mutual support has grown stronger as communities navigate the effects of war.⁹⁹ This communal sense of solidarity also extends to members of the LGBTQIA+ and PLHIV communities, whose experiences are generally shaped by everyday social interactions rather than being solely defined by their sexual orientation, gender identity, or health status. However, individuals whose sexual orientation, gender identity, or HIV status is not publicly known still voice concerns about potential stigma if their identity or status becomes public.¹⁰⁰

In many places, residents collaborate to clear debris after air attacks, repair windows, and help those unable to evacuate. Crisis situations – such as sudden shelling or changes in curfew – often prompt collective actions aimed at preserving stability and safety. In oblasts affected by shelling, for example, people noted joint efforts to clean basements, install handrails, and establish informal shelters. Residents of smaller towns tend to experience greater trust among neighbors, while those in larger cities and more diverse urban areas sometimes see more distant relationships.

While social unity in Ukraine has generally increased during the war, there have been challenges.¹⁰¹ Some participants across the different oblasts expressed unease about displaced people settling in their neighborhoods, citing discomfort with newcomers and apprehensions about potential informants who could share sensitive information with hostile parties or authorities. Fears of information misuse lead many to avoid discussing sensitive topics openly. Tensions between host communities and displaced people often arise due to differences in political views, religious beliefs, and language.¹⁰² In the initial stages of displacement, some displaced people faced hostility from locals, prompting them to take measures such as changing their license plates to match their new region of residence in an effort to avoid conflict.¹⁰³ Language barriers also influenced relocation decisions, with some Russian-speaking displaced people choosing to move to Odesa – where approximately 80 percent of the population speaks Russian –¹⁰⁴ to avoid the stigma they faced in other areas in Ukraine.¹⁰⁵ However, as displaced people have adapted to their new communities, these tensions have gradually lessened. According

99 UNDP. 2024. *Social Cohesion in Ukraine: Community Dynamics Amidst War*.

100 UNDP. 2024. *Social Cohesion in Ukraine: Tolerance, Trust, and Social Inclusion. Part II*.

101 IOM. 2025. *Displacement and Social Cohesion in Ukraine*.

102 IDMC. 2023. *Ukraine: Durable Solutions a Distant Prospect for Many IDPs*.

103 KII 8, NGO staff working with IDP

104 Rating Group. 2023. *Municipal Survey: May 2023*.

105 KII 15, INGO staff

to FGD participants across different regions, shared experiences of hardship, cooperation in rebuilding efforts, and mutual support have helped bridge early divides.

Despite these divisions, informal support networks remain vital. In some communities, residents share food and household goods with communities requiring assistance, including displaced people. However, tensions arising from perceived inequities in aid distribution can place strain on these relationships. Over time, collective endeavours – such as seasonal fruit-picking – have helped displaced people and residents connect on shared goals.

People with disabilities participating in the FGDs typically report positive relationships with those around them; many describe a strong sense of social inclusion within their immediate communities, marked by mutual trust, respect, and informal caregiving arrangements. Respondents across multiple oblasts shared that neighbors often assist with tasks such as fetching water, buying groceries, or navigating damaged infrastructure. While some mentioned challenges in accessing formal services or occasional feelings of marginalization in public spaces, day-to-day interactions with neighbors were overwhelmingly described as supportive and reliable. Although challenges persist, the war in Ukraine has, in multiple cases, sparked a heightened sense of solidarity, compelling individuals to unite to overcome adversities.

Socio-cultural and gender norms

Evolving gender roles

According to a World Vision study conducted across oblasts in Ukraine's four macro-regions,¹⁰⁶ nearly 40 percent of respondents have observed significant changes in gender dynamics since the start of the full-scale invasion.¹⁰⁷ Even before the war, women in many parts of Ukraine were increasingly assuming both professional and domestic responsibilities, often managing household finances, running errands, and caring for children independently. The war in Ukraine has accelerated this shift, creating new job opportunities for women in traditionally male-dominated fields like driving, delivery, and security. This change is largely driven by the mobilization of many men for military service or their departure from the country. Women are therefore increasingly both primary caregivers and de facto breadwinners, compounding the emotional and physical pressures they face.¹⁰⁸ Nonetheless, some FGD participants in the Dnipropetrovsk oblast noted that women are finding ways to maintain social connections through community

106 Ukraine's four macro-regions are: Kyiv + North, West + Centre, East, and South.

107 World Vision. 2024. *Gender-Based Violence in Ukraine Amid War: Assessment Report*. (Sample: 2,800 respondents)

108 KII 20, working group staff

groups and clubs, which provide mutual support and foster a sense of solidarity.¹⁰⁹ On the other hand, men who remain at home due to fear of conscription and limited job opportunities often take on additional household responsibilities, such as cooking and cleaning, reflecting a broader shift in traditional gender roles.

Women's influence also extends further into family decision-making, particularly around household budgets and food purchases. According to WFP's 2023 annual country report, 45 percent of households receiving cash-based transfers from WFP reported that women alone decide how to allocate the aid, compared to only eight percent for men. Similarly, among households receiving in-kind assistance, 53 percent reported that women determine how to distribute the aid, versus nine percent for men.¹¹⁰

Protection risks

Conflict Sensitivity

Disputes over humanitarian aid are relatively rare in Ukraine, and all participants consistently acknowledged that assistance is provided free of charge. However, tensions arise when certain groups perceive aid distribution as unfair or influenced by political motivations. While humanitarian organizations are generally seen as neutral, some participants voiced frustration with feeling overlooked or excluded from receiving aid. If community members feel they are not consulted or fairly targeted, mistrust and tensions can escalate, weakening the perceived neutrality of humanitarian actors.

Crowding at distribution sites can sometimes lead to short-lived conflicts, though better coordination and scheduling have helped reduce these issues. Nonetheless, overcrowding remains a significant conflict sensitivity concern because it fuels frustration not only among beneficiaries themselves but also toward aid organizations. FGD participants described chaotic lines, inadequate crowd management, and long waiting times with no guarantee of receiving aid, all of which contributed to tension and complaints.¹¹¹ While participants did not always clarify whether these challenges reflected current conditions or past experiences, some noted that the organization of aid distribution has improved since the start of the full-scale invasion.¹¹²

Additionally, the perception of politicized aid distribution is a significant conflict sensitivity risk. In contexts where local authorities or influential individuals

109 FGD 10, with women (18-59), Dnipropetrovsk oblast

110 WFP. 2024. *Ukraine Annual Country Report 2023*.

111 FGD 12, with older women (59+), in Dnipropetrovsk oblast; FGD 14, with IDPs, in Dnipropetrovsk oblast; FGD 20, with women (18-59), in Odesa oblast; FGD 21, with older women (59+), in Odesa oblast; FGD 23, with Older men (59+), in Mykolaiv oblast; FGD 40, with IDPs, in Vinnytsia oblast

112 FGD 6, with men (18-59), in Kharkiv; FGD 10, with women (18-59), in Dnipropetrovsk

have control over beneficiary lists or distribution processes, communities may view access to aid as dependent on personal or political connections. These concerns become even more pronounced during program transitions – such as downsizing or shifting to development-focused support – when fewer people receive assistance and targeting becomes stricter. Without clear communication and accountability, this can heighten social tensions and undermine trust in both humanitarian actors and local governance. Additionally, the visible presence of military representatives at some distribution points has deterred men from accessing assistance due to fears of conscription.¹¹³

Safety, dignity, and do no harm

Despite ongoing efforts to improve accessibility, many still face challenges in obtaining aid safely. Overcrowded distribution points, long waiting times, and security risks – such as shelling in regions affected by active hostilities – create dangerous conditions for collecting assistance. To address this, some organizations have introduced mitigation measures such as scheduled aid pick-up appointments, door-to-door deliveries (especially for vulnerable individuals and those in high-risk areas), the use of services like Nova Poshta, a Ukrainian postal and courier company, and increased staffing at distribution centres to ensure safer and more efficient distribution processes. In addition to improving safety, many FGD participants emphasized that these more discreet and flexible distribution methods not only reduce exposure to physical risks but also promote more dignified access to aid, particularly for individuals who may feel stigmatized or physically unable to queue at public distribution points.

Generally, people's sense of safety is shaped by the ongoing war. In oblasts affected by active hostilities, such as Donetsk and Kharkiv, frequent air alarms and a shortage of bomb shelters intensify safety concerns. In oblast like Zaporizhzhia, the lack of legal aid and psychological support further contributes to the sense of vulnerability. Recognizing this, participants also called for mental health support alongside material aid, emphasizing the need for psychological care to help people cope in an environment where both security and stability remain in short supply.

Mistrust in law enforcement was also highlighted by FGD participants as a factor undermining their sense of safety. FGD participants residing in a temporary settlement also reported feeling unsafe due to its proximity to abandoned buildings with drug activity and homeless populations, noting that their repeated calls to construct a fence around the area had been ignored by authorities.¹¹⁴

113 KII 20, working group staff

114 FGD 33, with IDPs, Kyiv Oblast

Meaningful Access

Across all the assessed community groups, cash assistance is overwhelmingly preferred as it allows people to prioritize their most urgent needs, including food, medicine, utilities, and rent. Participants emphasized that financial aid offer greater flexibility and autonomy, helping them manage expenses over time and reducing their dependence on in-kind distributions, which usually have to be collected in person. However, in-kind assistance remains essential in specific contexts – particularly in regions affected by active hostilities, rural communities, and oblasts with limited access to markets, clean water, or heating. In some situations, the form of aid is less important than its availability; for example, displaced people often rely on housing support – whether in the form of financial assistance to cover housing costs or access to physical shelters – as they frequently face difficulties in finding and affording suitable accommodation.

Physical barriers also limit access, especially in remote or conflict-affected areas, where reduced public transit, damaged infrastructure, and security risks make it difficult to reach assistance points and essential services. According to participants residing in smaller frontline towns such as those located in the Donetsk oblast, public transportation services have been reduced, forcing people to travel long distances to reach basic services such as markets, healthcare facilities, and aid distribution points. People living in rural areas, including internally displaced people, are particularly impacted by the discontinuation of public transport services, which has limited their ability to reach humanitarian centres.

In some frontline cities, such as Myrnohrad in Donetsk oblast, a 20-hour curfew severely restricts movement. Other cities in Kharkiv and Kherson oblasts also impose long curfews, sometimes lasting up to 16 hours. To improve access, an NGO has launched social bus services in communities across Donetsk oblast, ensuring that people in vulnerable situations can access essential services and assistance.

Limited digital literacy among certain population groups – such as older people or individuals with disabilities that may impact their ability to navigate or complete application forms – hinders access to assistance. This issue is further exacerbated by short application windows, driven by high demand and limited supply of support. Furthermore, individuals are often required to provide extensive documentation to prove eligibility, which can be difficult to obtain. This poses a particular challenge for displaced people, who may have lost or left behind essential documents, and for Roma individuals, who often lack official documentation altogether. When registering for humanitarian assistance, participants also expressed concerns over data security. Many participants across different oblasts hesitate to share personal information – such as passport numbers, tax IDs, or bank details – due to fears of fraud, misuse, or lack of

transparency. They are more willing to provide this information if it guarantees assistance, but feel uncomfortable sharing it when there is no certainty of receiving aid.

Service disruptions due to insecurity add another layer of exclusion. FGD participants in areas affected by hostilities described how frequent shelling, power outages, and disrupted supply chains have hindered access to basic services like healthcare, clean water, and solid fuel for heating. These conditions disproportionately affect the older people, persons with disabilities, and residents in remote rural villages, many of whom lack transportation or digital access to humanitarian registration systems. Finally, in areas affected by active hostilities, access to cash assistance is further constrained by logistical disruptions, with participants frequently reporting difficulties receiving payments due to damaged ATMs.



The first obstacle is that he cannot receive it [aid] because he has no information, he does not know where to get it at all... it is so difficult to fill out the forms and the time is very limited. Because our computer literacy is so low, we can't do it in time.

FGD 19, with older men (60+), Zaporizhzhia oblast



Accountability

When it comes to the source of aid, many respondents express a strong preference for assistance from international organizations over local authorities or organizations (CSOs/NGOs). International organizations are seen as more transparent, reliable, and better resourced, whereas local entities are often viewed as inefficient, slower in response, or prone to favoring certain individuals or groups over others in aid distribution. Participants across different regions described instances where local actors appeared to prioritize friends, relatives, or those with personal connections, leading to perceptions of nepotism and corruption. In contrast, respondents felt that international organizations follow more standardized procedures and are less vulnerable to local power dynamics or manipulation.

For instance, In Vinnytsia, participants described scandals where local leaders appropriated international humanitarian aid, distributing it under their own names to boost their political profiles¹¹⁵ However, it is worth noting that some of the

115 FGD 39, with older men (60+), Vinnytsia oblast

claims raised during data collection are based on personal accounts and are not always supported by concrete evidence.



There was a scandal with humanitarian aid; ... Groysman hid some humanitarian aid. This aid was distributed exclusively to certain people. It included hygiene products. He received it from some international organization but presented it as his assistance.

FGD 39 with older men (60+), Vinnytsia region



Effective communication is also a key accountability concern. Poor information sharing between aid providers and the affected people can lead to frustration and complicate access, making it difficult for people to understand where to go, how to apply, and who is eligible to receive support. In Odessa, for instance, participants reported that information about WFP assistance is unclear, leaving many of the participants unsure about where to register.¹¹⁶

Accountability to affected people

Participation in decision-making

In contrast to humanitarian organizations' efforts to distribute information more widely, actual decision-making about aid distribution remains concentrated among key institutions. International humanitarian organizations apply their own targeting frameworks, which are informed by needs assessments and consultations with relevant Ukrainian authorities. Meanwhile, government bodies and community-level organizations (village councils, housing cooperatives, heads of hromada) support allocation by identifying and providing lists of individuals eligible for aid. In Donetsk and Kharkiv oblast, for example, territorial centres and city councils coordinate with social welfare departments to identify beneficiaries. In Lviv, the Social Protection Office evaluates individuals' eligibility based on factors like displacement or disability status.

Community members who participated in this study reported feeling excluded from meaningful participation in decision-making processes. This perception is echoed by a CARE survey conducted in Dnipropetrovsk, Kharkiv, Mykolaiv, and Odesa, which found that 88 percent of men and women respondents were rarely or never consulted by aid organizations about their needs.¹¹⁷ It is important to note, however, that while these specific individuals may not have been consulted,

116 FGD 20, with women (18-59), Odessa oblast

117 CARE International. 2023. *Rapid Gender Analysis: Ukraine*. (n=611)

many humanitarian organizations do rely on needs assessments involving selected samples. As such, although some voices were not included, others may have contributed to the design of aid programs.

Respondents expressed a desire for greater transparency and inclusivity in aid distribution decisions. A common concern was the lack of clear eligibility criteria, leading to frustration and perceptions of unfairness. Many participants reported that they did not understand why some individuals received assistance while others did not. To address this, they suggested publicly clarifying eligibility requirements, increasing the overall quantity of aid to reduce denials, and implementing pre-registration or scheduling systems to minimize overcrowding. Additionally, some participants in Vinnytsia region described feeling humiliated or stigmatized when forced to wait in long queues for aid items they may not need but feel obligated to accept. As a solution, they proposed the introduction of small “free choice” shops where recipients can select only the items they require, as well as sheltered distribution points to protect them from harsh weather conditions.



It's awful. You feel like an animal in a zoo. They give you something, you come, and you might not even like it or need it, but you go out of necessity.

FGD 38, with women (18-59), Vinnytsia oblast



Information Provision

Humanitarian actors use a combination of digital and traditional channels to reach broader populations across Ukraine. Many people learn about aid opportunities through social media platforms such as Facebook, Instagram, Telegram, and TikTok. For example, Telegram is used by organizations to communicate with each other, as well to post information about the possibility of providing different types of assistance.¹¹⁸ However, direct communication through phone calls, instead of relying solely on digital platforms, is seen as essential for reducing misunderstandings and frustration between potential beneficiaries and aid providers. It ensures clearer information and enables more human interaction, allowing questions to be asked and answered effectively. Additionally, commonly used communication methods – such as word of mouth, SMS notifications, radio announcements, billboards, local councils, and community networks – remain crucial, particularly in smaller and frontline towns with limited internet coverage and among hard-to-reach groups who do not always trust outsiders.¹¹⁹

118 KII 8, NGO staff working with IDP

119 KII 15, INGO staff

These preferences and constraints have led local actors to adopt a variety of context-specific communication strategies across different regions. In Dnipropetrovsk oblast, for example, the city council website and broadcast media (radio and TV) are key platforms for announcements about aid distribution or transport options.¹²⁰ In contrast, the Zaporizhzhia oblast local authorities and NGOs sometimes visit villages directly to register potential recipients. Meanwhile, in Vinnytsia oblast, the city council uses billboards and a hotline system to help bridge information gaps.



Digital literacy is a significant factor in Ukraine's humanitarian landscape. As a digitally connected country, Ukraine relies heavily on platforms like Telegram for sharing information about services and distributions.

KII 15 with IO representative



Community Feedback Mechanism

Respondents widely reported using social media platforms such as Viber, Telegram, and Facebook to provide feedback to aid providers, valuing them for their ease of use and anonymity. While in-person feedback remains an option, many avoid it due to long wait times when trying to speak with a representative, security risks, or fear of mistreatment. For people in vulnerable situations, specific feedback channels are adapted to their needs. Within Roma settlements, displaced communities, and groups with limited digital literacy, hotline calls, and personal visits serve as the main avenues for providing feedback and raising concerns.

When communicating with assistance providers for information or clarification, most people prefer using hotlines which offer a direct channel for submitting requests and complaints, as well as social media, local community leadership, or in-person visits to the organization's office. However, participants noted that hotlines lack the anonymity provided by social-media feedback streams. Some FGD participants also called for more frequent surveys and direct outreach to ensure aid providers remain accountable and responsive to evolving needs.

Ultimately, despite widespread data-privacy worries, communities continue to seek ways to make their voices heard. Strengthening trust, instituting transparent data-protection measures, and offering multiple avenues of communication – digital, telephone-based, and in person – can foster more inclusive and effective feedback processes.

120 FGD 11, with male (18-59), Dnipropetrovsk oblast

Community-Specific Findings

Women (18-59)

Demographics and current humanitarian situation for women (18-59)

In 2021, Ukraine's population was approximately 41.4 million, with women comprising 53.6 percent of the total. However, due to the ongoing war and its humanitarian impact, the number of Ukrainians residing within national borders has decreased, with estimates indicating that 37.9 million people remain in the country in 2024, of whom 53.5 percent (approximately 20.26 million) are women.¹²¹ The full-scale invasion has triggered significant demographic shifts, including mass displacement, migration, and changes in household structures. According to REACH, 64 percent of households are now female headed,¹²² as many men have been conscripted into military service. Additionally, women make up 72 percent of those registered as unemployed and 56 percent of the country's 3.7 million internally displaced people, highlighting the disproportionate impact of the full-scale invasion on women.¹²³



With more men conscripted, women face growing caregiving demands. They now manage not only household duties but also external tasks. This responsibility extends to caring for returning veterans with disabilities and assisting with evacuations, especially for older family members (from South and East).

KII 14 with IO representative



Provision and access to basic services and resources

Women from FGDs aged 18 to 59 identified their primary needs as healthcare (particularly for children), childcare, access to utilities or support for utility costs, employment, and financial security. According to REACH, the most frequently reported priority needs among female-headed households (FHHs) include medicine (39 percent), food (37 percent), healthcare (28 percent), and fuel for heating (17 percent), reflecting the urgent challenges they face in meeting basic necessities.¹²⁴

121 UNFPA. 2024. *Ukraine Population Data*.

122 REACH. 2024. *Multi-Sector Needs Assessment: Gender, Age, and Disability Situation Overview*. (Sample 13,322 households)

123 UN Women. 2024. *Women and Girls After Two Years of War in Ukraine*.

124 REACH. 2024. *Multi-Sector Needs Assessment: Gender, Age, and Disability Situation Overview*. (Sample 13,322 households)

As reported by several women during FGDs, government clinics often have long waiting times, while private healthcare remains unaffordable for people in vulnerable situations. The lack of medical specialists, particularly in pediatrics, in smaller towns such as those in Kharkiv and Zaporizhzhia oblasts forces residents to travel long distances for treatment.¹²⁵



I have a younger child who needs a specialist, a paediatric neurologist. But unfortunately, there is none in our town. We had to look for one in Kharkiv.

FGD 5, with women (18-59), Kharkiv oblast



Women who participated in FGDs across all oblasts also reported that access to utilities and their costs are among the most pressing challenges they face. Frequent power outages disrupt daily life, affecting heating systems, water access, and remote education. According to a study conducted by REACH, 17 percent of women consider fuel for heating one of their most urgent needs.¹²⁶



All residents were transferred to individual heating. The price is high, people have no jobs in the city because everything is closed, we are in a combat zone. Secondly, there will be no electricity because nearly the entire energy infrastructure is damaged.

FGD 10, with women (18-59), Dnipropetrovsk oblast



Limited access to childcare significantly impacts women's ability to work, forcing many to stay at home to care for their children. In several oblasts, public kindergartens remain closed or operate at reduced capacity, while private childcare options are often too expensive for most families.



Employment opportunities for women with small children are limited, because you have no institution to take your kids.

FGD 16, with women (18-59), Zaporizhzhia oblast



125 FGD 5 and 16, with women (18-59), Kharkiv and Zaporizhzhia oblasts

126 REACH. 2024. *Multi-Sector Needs Assessment: Gender, Age, and Disability Situation Overview*. (Sample 13,322 households)

Provision and access to assistance

Women participating in FGDs often reported relying on a mix of external aid, informal work, and support from family networks to meet their basic needs. Although 94 percent of women report no barriers to accessing food assistance,¹²⁷ strict eligibility criteria and paperwork that is difficult to obtain are barriers for some individuals to access assistance.

Women FGD participants widely expressed a preference for cash assistance or vouchers rather than in-kind aid, stating that financial support provides greater autonomy in covering essentials such as rent, utilities, and clothing. Available data shows that women are more likely than men to apply for cash assistance,¹²⁸ with single mothers showing particularly high demand—70 percent apply for financial aid, compared to the overall average of 54 percent.¹²⁹ Despite widespread appreciation for food distributions, some women note inefficiencies, like receiving the same staple items repeatedly while lacking basics like oil or margarine. In areas where organizations offer both cash assistance and food packages, most recipients strongly prefer having the flexibility to choose between the two options.

Despite ongoing aid efforts, significant gaps remain, especially in the distribution of hygiene products.¹³⁰ These items are often provided inconsistently, with many women suddenly losing access to them, as they may not be able to afford them on their own. Single mothers and those without formal employment face additional barriers, as they often do not qualify for state benefits and rely heavily on limited humanitarian aid.



[before the war] I could buy and eat, buy clothes for a child, pay utility bills... When they give 3 kg of flour as humanitarian aid, I would rather buy 1 kg of flour and a jar of pate. And the rest of the money, I would leave for utility services.

FGD 26, with women, Kherson oblast



127 WFP. 2024. *Ukraine Annual Country Report 2023*.

128 CARE International. 2023. *Rapid Gender Analysis: Ukraine*.

129 KII 19, INGO staff

130 FGD 16, with women (18-59), Zaporizhzhia oblast

Economy and livelihoods

Access to jobs

Despite the availability of jobs in sectors such as beauty services, factory work, social services, the food industry, and trade, female participants reported structural barriers – such as limited qualifications and inadequate childcare options – that prevent them from securing stable employment. Some women, adapting to the changing economy, create online content on platforms like YouTube to earn extra income.¹³¹

Finding a job has become increasingly difficult in many areas due to the destruction of facilities. On average, the unemployment rate of women aged 18–50 is eight percent. Among displaced women (18–59), the unemployment rate is 20 percent, while the employment rate is 45 percent. For non-displaced women, the unemployment rate is seven percent, while the employment rate is 65 percent. Additionally, 18 percent are engaged in housework, 6 percent are living with disabilities, and three percent are non-working students. Among returnees, the unemployment rate is 13 percent, while the employment rate is 63 percent.¹³² In some rural areas, available jobs offer very low wages, making them unattractive despite widespread unemployment. Women in urban areas have a higher employment rate of 66 percent, compared to 59 percent for women in rural areas, further highlighting geographical disparities.¹³³

Persistent wage inequality also undermines women’s financial stability, with women earning almost 19 percent less than men for equivalent work.¹³⁴ Despite many women increasingly becoming the primary breadwinners and taking on roles traditionally dominated by men, they continue to face significant pay disparities. This forces them to work more – often in multiple jobs or in informal sectors – not just to cope with rising costs, but to compensate for their lower earnings compared to men. The wage gap is most pronounced among women aged 25–44,¹³⁵ who, facing high unemployment and limited formal job opportunities (especially for unskilled workers), often turn to informal or part-time work such as housekeeping, sewing, baking, or collecting and selling recyclable materials – such as scrap metal, plastic bottles, glass, and cardboard – to make ends meet. As economic hardship deepens, some women face heightened risk of exploitation, including being forced into survival sex work or becoming targets of human trafficking.¹³⁶

131 FGD 20 (2), with women (18-59), Odessa (2) oblast

132 REACH. 2024. *Multi-Sector Needs Assessment: Gender, Age, and Disability Situation Overview*. (Sample: 13,322 households)

133 Ibid

134 Ibid

135 Ibid

136 KII 14, IO Staff



Reduced economic opportunities have also led to an increase in survival sex work and trafficking... This has also resulted in a rise in online recruitment for exploitative purposes.

KII 14 with IO representative



Human, social, natural, physical, and financial capital

For a significant number of women, financial assistance (whether from government programs or humanitarian organizations) remains their primary income source, yet these benefits often fall short of meeting basic expenses. Female-headed households (FHHs) are particularly vulnerable with 21 percent relying on government or social benefits¹³⁷

When families do receive support from relatives living abroad or in safer parts of Ukraine, the assistance varies significantly, ranging from consistent financial transfers to sporadic in-kind support. According to a study conducted by REACH, nine percent of women receive remittances.¹³⁸ Some enjoy consistent remittances covering food, utilities, and other daily costs, while others receive no external aid at all. In Kherson, a woman described how her husband's continued work abroad secures them a stable income,¹³⁹ whereas participants in other oblasts rely sporadically on parcels of food and clothing from friends.



Those of my friends, acquaintances who left...they help with clothes. My friend sends me her son's things from Germany.

FGD 31, with women (18-59), Kyiv oblast



¹³⁷ REACH. 2024. Multi-Sector Needs Assessment: Gender, Age, and Disability Situation Overview. (Sample: 13,322 households)

¹³⁸ Ibid

¹³⁹ FGD 26, with women (18-59), Kherson oblast

Community structure and meaningful participation

Information and knowledge management

Given their numerous responsibilities and limited free time, women need concise, direct, and easily accessible communication about available assistance and services. According to International Rescue Committee (IRC) data, 72 percent of women surveyed indicated a preference for receiving information about assistance through phone calls or SMS, 31 percent through direct contact with IRC staff, and 21 percent through updates from local authorities.¹⁴⁰ This highlights the need for accessible, varied communication methods – ranging from offline channel to centralized digital platforms – so that women can promptly learn about available assistance. In line with these preferences, participants across multiple oblasts suggested expanding offline communication via billboards, noticeboards, and community meetings; offering transparent information through trusted local sources such as city councils, religious organizations, and verified NGOs; enhancing online registration systems to be more user-friendly; and providing hotlines or SMS notifications for those without regular internet access.

Socio-cultural and gender norms

Evolving gender roles

Beyond demographic changes, the war in Ukraine has altered gender roles and the labor market. With many men either serving in the military or displaced, women have increasingly stepped into roles traditionally dominated by men, particularly in agriculture, transportation, and industry.¹⁴¹ This shift has not only increased women's participation in the formal workforce but has also expanded their responsibilities in community leadership, household financial management, and essential service sectors. While these changes highlight women's resilience and adaptability, they also underscore the long-term social and economic challenges of evolving gender roles in households and workforce participation amid an ongoing war. Some women participating in the FGDs reported feeling underprepared for these new responsibilities, often lacking the necessary training or skills—such as IT or foreign languages—to make a successful career transition. Although some have pursued online courses or support through employment centers, access remains inconsistent and hampered by financial, informational, and structural barriers.

140 IRC. 2024. *Ukraine Multisectoral Need Assessment Report – March 2024*. (Sample: 732 respondents)

141 UN Women. 2024. *Women's Economic Empowerment in Ukraine During Russia's Invasion*.



Women bear the burden of everything. Nowadays, a woman is a mother, father, grandmother, grandfather, and neighbour.

FGD 38, with women (18-59), Vinnytsia region



Protection risks

Gender-based violence

GBV remains a serious issue in Ukraine, exacerbated by conflict-related hardships. A study by World Vision found that the main triggers of GBV were poor living conditions, cited by 42 percent of respondents, and the character of the perpetrators, cited by 40 percent of respondents. Women are most commonly abused by current partners, with 23 percent of those who reported psychological violence¹⁴², 27 percent of those who experienced economic violence¹⁴³, and 20 percent of those subjected to physical violence¹⁴⁴ identifying their partner as the perpetrator. Among the 12 percent of respondents who reported experiencing sexual violence, the most common perpetrators were male relatives (36 percent) and partners (27 percent).¹⁴⁵ In cases of sexual harassment, which were reported by 54 percent of respondents, strangers were identified as the most frequent perpetrators, accounting for 41 percent among those experiencing such abuse.¹⁴⁶

Many survivors do not seek help due to stigma or lack of trust in institutions – 27 percent believe reporting will not change anything, while 24 percent see it as a private matter.¹⁴⁷ As a result, only 24 percent of survivors of physical or sexual violence seek help. Limited access to mental health and psychosocial support (MHPSS), particularly in rural areas, hinders recovery, with 32 percent of women

142 Psychological violence: Emotional abuse such as insults, humiliation, threats, or intimidation

143 Economic violence: Control or restriction of access to financial resources or economic independence

144 Physical violence: Acts involving physical harm, including hitting, pushing, or other forms of bodily harm

145 Sexual violence: Non-consensual sexual acts

146 Sexual harassment: Unwanted sexual comments, gestures, or advances

147 World Vision. 2024. *Gender-Based Violence in Ukraine Amid War: Assessment Report*. (Sample: 2,800 respondents)

aged 19–59 reporting a lack of services.¹⁴⁸ Additionally, as noted by a UNFPA staff member, the number of gender-based violence complaints reported to the police has significantly increased since the start of the international armed conflict. However, it is important to highlight that reporting GBV incidents to the police remains uncommon in Ukraine, typically occurring only as a last resort in life-threatening situations.¹⁴⁹

Various types of assistance are provided to survivors of GBV. Women who have experienced GBV frequently express a need for food assistance, a recurring request identified by UNFPA in GBV shelters. This need is especially urgent as many women arrive at shelters with no belongings, having fled their homes without money or food. While humanitarian assistance is essential for survivors, it must be provided carefully to avoid exacerbating their risk of further harm.



For cash assistance provided through GBV case management, we [Member of IO] must carefully consider the potential risks that this support could create. For example, if a woman returns home with cash, and the perpetrator is still present, there is a risk that the situation could escalate and become dangerous for the victim of GBV.

KII 12 with IO representative



In line with these broader GBV challenges, the 2023 Sexual Exploitation and Abuse Risk Assessment Report highlights that ongoing conflict, displacement, and socio-economic pressures significantly heighten SEA risks for marginalized women and girls. Underreporting persists due to stigma and limited awareness of reporting mechanisms, while frontline relief activities and cash assistance programs can inadvertently magnify risk without robust prevention measures.¹⁵⁰

To address growing humanitarian needs, a harmonized response framework is being developed by the Gender-Based Violence Area of Responsibility (GBV AOR) and the protection cluster. The first phase prioritized emergency in-

148 CARE International. 2023. *Rapid Gender Analysis: Ukraine*. (Sample: 611 respondents)

149 KII 18, IO staff

150 PSEA Network Ukraine. 2024. *Ukraine Sexual Exploitation and Abuse Risk Assessment Report 2023*.

kind or cash assistance for GBV survivors, low-income families, and internally displaced people no longer receiving state allowances, while the second phase focuses on linking affected individuals to sustainable livelihood opportunities.¹⁵¹ Given the vulnerability of GBV survivors, immediate financial support is prioritized over long-term employment solutions,¹⁵² and the Ukraine PSEA Network emphasizes the importance of integrating SEA risk mitigation – such as clear reporting pathways, staff training, and strong coordination with GBV services – into these assistance efforts to ensure survivors receive comprehensive, survivor-centered support.¹⁵³

Men (18-59)

Demographics and current humanitarian situation

In 2024, Ukraine's population is estimated to be 37.9 million, with men accounting for approximately 46.5 percent, or 17.6 million.¹⁵⁴ The ongoing war prompted the conscription of a substantial portion of the male population, requiring men aged 25 to 60 to register for military service. Consequently, there is a noticeable absence of men in public spaces, as many have been conscripted or attempt to avoid conscription.¹⁵⁵

The war in Ukraine has contributed to a significant number of male casualties. According to Ukraine's President, as of February 2025, more than 45,000 men have been killed on the battlefield, and over 390,000 soldiers have been wounded.¹⁵⁶

Provision and access to basic services and resources

Men from the Mykolaiv oblast participating in FGDs reported needing access to clean and reliable water sources, describing it as one of the region's most pressing and unresolved issues, with tap water deemed unsafe even for washing.¹⁵⁷ Participants have also identified access to quality education for their children, adequate medical services, and essential medicines as essential needs. They also emphasized the need for functional transportation and basic necessities such as food, clothing, and heating materials as the ongoing shelling, curfews, the risk of conscription, and economic hardships further limit their ability to secure

151 KII 13, cluster staff and KII 18, IO staff

152 KII 18, IO staff

153 PSEA Network Ukraine. 2024. *Ukraine Sexual Exploitation and Abuse Risk Assessment Report 2023*.

154 UNFPA. 2024. *Ukraine Population Data*.

155 FGD 11. Men (18-59), Dnipropetrovsk oblast

156 The Kyiv Independent. 2025. Over 45,000 Ukrainian soldiers killed since start of war, Zelensky says

157 FGD 22, Men (18-59), Mykolaiv oblast

livelihoods and access essential resources.

Many men avoid registering for assistance or visiting public distribution centres due to fears of conscription, as concerns over sharing personal information and being identified discourage age participation. This reluctance limits their access to essential support, leaving them without much-needed aid. To address this issue, participants suggested implementing a family-based registration system, allowing one household member to submit documents and collect aid on behalf of the family, reducing individual exposure. Additionally, increasing transparency about how personal data is used and protected could help alleviate fears and build trust, ensuring that those in need feel safer accessing humanitarian assistance.

Economy and livelihoods

Access to jobs

In Ukraine, 63.9 percent of men rely solely on paid work as their income source, often avoiding assistance programs due to fears associated with registration.¹⁵⁸ However, they face significant challenges in the labor market, with 25 percent of men aged 26-50 engaged in unstable employment, compared to 15 percent of women in the same age group. Among men aged 18-25 and 26-50, employment stands at 59 percent and 84 percent, respectively. However, 19 percent of men aged 18-25 hold unstable jobs,¹⁵⁹ compared to 25 percent of men aged 26-50. According to REACH, this difference is linked to concerns about conscription, which influence employment stability among men in these age groups.¹⁶⁰ Employment conditions also vary by displacement status, with returnee men aged 18-29 having the highest employment rate (83 percent) compared to displaced (68 percent) and non-displaced men (78 percent).¹⁶¹

Beyond these employment challenges, the risk of conscription has become a significant barrier to securing formal work, as job opportunities for men are often tied to their military service status.¹⁶² To be officially employed, men are required to obtain documents from Territorial Centres of Recruitment (TCR). This process necessitates registration with the TCR or visits to their offices, which many men avoid due to fears of conscription. Employers are also hesitant to hire men, as they may face conscription if regulations change, further restricting their employment

158 CARE International. 2023. *Rapid Gender Analysis: Ukraine*.

159 Unstable jobs: Unstable, low-paying, and insecure forms of employment

160 REACH. 2024. *Multi-Sector Needs Assessment: Gender, Age, and Disability Situation Overview*. (Sample: 13,322 households)

161 REACH. 2024. *Multi-Sector Needs Assessment: Gender, Age, and Disability Situation Overview*. (Sample: 13,322 households)

162 CARE International. 2023. *Rapid Gender Analysis: Ukraine*. (Sample: 611 respondents)

opportunities. To navigate these challenges, many men resort to unofficial employment, which offers income but lacks stability, legal protections, and social benefits.



What we said was that a lot of jobs require you to go to the military registration and draft office first. And that's a problem.

FGD 11, with men (18-59), Dnipropetrovsk oblast



Human, social, natural, physical, and financial capital

Male-headed households rely less on remittances (four percent) and social benefits (21 percent) compared to female-headed households.¹⁶³ Some male participants reported receiving support from friends and relatives, including those living abroad, though this depends on household composition and available social networks. While external humanitarian assistance remains a crucial lifeline, many households also rely on savings, maintaining livestock, or engaging in gardening to secure food. Others adjust by cutting non-essential expenses, taking on part-time work, or participating in local bartering networks to exchange goods or sell homemade products.

Protection risks

Community safety

Male FGD participants expressed heightened feelings of insecurity due to the persistent risk of mobilization. Men consider avoiding conscription because of inadequate material support (66 percent), lack of required training before assignment to the unit (64 percent), and the possibility of being injured or killed (64 percent).¹⁶⁴ This fear greatly limits their ability to move freely and maintain normal routines, often forcing them to remain in hiding to avoid being drafted.



How can you be protected as you walk around the city and look around to avoid encountering a certain category of military personnel?

FGD 11, with men (18-59), Dnipropetrovsk oblast



¹⁶³ REACH. 2024. *Multi-Sector Needs Assessment: Gender, Age, and Disability Situation Overview*. (Sample: 13 322 households)

¹⁶⁴ Texty.org.ua. 2024 Мобілізація: що відлякує і що мотивує. (Sample: 400 respondents)

In oblasts that have been occupied, men face a high risk of landmine explosions, especially during agricultural work, which remains a predominantly “male” sphere. According to Help Women Agro in Ukraine, before the full-scale invasion about 70 percent of the total number of farmers and small and medium agricultural producers in Ukraine were men.¹⁶⁵ These dangers are compounded by limited access to critical safety information, such as how to identify and avoid landmines, and emergency procedures for handling unexploded mines. The threat is particularly acute in oblasts affected by active hostilities like Kherson, where the risk of unexploded ordnance is widespread.



This knowledge gap is particularly concerning in regions like Kherson, where unexploded ordnance is prevalent. This poses heightened risks for individuals working in agriculture, as there has been an increase in injuries due to mine-related incidents as people continue working on to fields despite these dangers to secure livelihoods.

KII 14 with IO representative



Gender-based violence

A World Vision study found that 65 percent of male participants had experienced psychological violence. Among them, 24 percent said it came from their current partners, 23 percent from colleagues, and 22 percent from female friends. Economic violence was reported by 53 percent of men, with 30 percent attributing it to current partners and another 30 percent to female relatives. Physical violence affected 33 percent of respondents, with 27 percent identifying female colleagues as the perpetrators. Female colleagues were also most often named as the source of sexual harassment (24 percent) and sexual violence (29 percent). In total, 36 percent of men reported being harassed, and 9 percent said they were victims of sexual violence.¹⁶⁶

NGO representatives have noted a growing number of men becoming victims of conflict-related GBV – particularly sexual violence and torture – in occupied territories and regions affected by active

¹⁶⁵ AgroPortal. 2023. *В українському агросекторі все ще мало жінок. Про компанію, яка руйнує стереотипи*

¹⁶⁶ World Vision. 2024. *Gender-Based Violence in Ukraine Amid War: Assessment Report*. (Sample 1400 respondents)

hostilities. This is especially concerning given the risk of abduction by the Russian army in these areas. Addressing conflict-related GBV against men remains particularly challenging due to stigma, shame, and limited monitoring mechanisms that hinder reporting and intervention. Additionally, many men feel excluded from awareness campaigns, which primarily focus on women, and some fear that coming forward may lead to forced military service.¹⁶⁷

There is also a pronounced lack of shelters, crisis rooms, and specialized support services for male survivors. Facilities intended for women survivors explicitly prohibit men, leaving them with few safe options. Furthermore, there is a shortage of medical and psychological professionals trained to handle the specific needs of male survivors



All services such as shelters, crisis rooms, and daycare centres are targeted and are safe spaces for women and girls, and males are not allowed there. So, we don't have specialized services. Meanwhile, 10 percent of our survivors are males. Most of them experience psychological violence, but sometimes they require relocation, and there is no service to support this. There are also survivors of conflict-related sexual violence, and we don't have specially trained doctors who can provide relevant care for male survivors of rape or other types of sexual violence.

KII 12 with IO representative



Older people (59+)

Demographics and current humanitarian situation

Ukraine has one of the highest proportions of older people in Europe, with individuals aged 60 and above making up around a quarter of its population. Additionally, 37 percent of households in Ukraine are headed by individuals aged 60 or older.

Older people have been disproportionately affected by the war in Ukraine, accounting for 35 percent of civilian deaths in the first year of fighting. Beyond the direct impact of violence, they face a range of unique challenges including widespread poverty and an inability to meet basic needs, limited access to food, utilities, clothing, healthcare, and transportation, as well as age discrimination in the labor market. Additionally, many older individuals struggle with digital literacy and social isolation, which hinders their ability to access vital services and assistance.¹⁶⁸

Provision and access to basic services and resources

Older Ukrainians often rely on small pensions that barely cover everyday expenses, let alone medical costs. As of October 1, 2024, the average monthly pension in Ukraine was UAH 5,852 (approximately USD 147.3), with about 30 percent of pensioners receiving less than UAH 3,000 (USD 75.5).¹⁶⁹ Given these financial constraints, many older individuals identify access to medicine and healthcare as their top priority.¹⁷⁰

Although many areas have healthcare facilities, attacks on them have severely limited their capacities to provide care to older people.¹⁷¹ Mobile healthcare teams and pharmacies in rural areas provide some services, but they are often insufficient to meet the needs of older people. Even with price reductions for pensioners, the high cost of medicines and diagnostic tests remains a major barrier, forcing many to choose between medical care and other essential expenses. As a result, many older participants reported discontinuing necessary treatments because they simply cannot afford them.

Compounding these challenges, a shortage of medical professionals—including doctors and nurses—has led to the closure of some medical departments, causing overcrowding in those that remain operational. In oblasts such as Dnipropetrovsk

168 **World Bank. 2024. Ukraine - Third Rapid Damage and Needs Assessment (RDNA3) (Report No. P180174).**

169 Vox Ukraine. 2024. *Pension System in Ukraine: Citizens' Perspective.*

170 WFP. 2024. *Ukraine Annual Country Report 2023.*

171 Ibid

and Mykolaiv, certain healthcare facilities also lack essential resources, such as ambulances, making it even more difficult for people in remote areas to access timely medical care.¹⁷²

Provision and access to assistance

Older men and women face heightened challenges due to isolation and lack of financial resources, with their primary concerns being access to utilities, food, and hygiene products, including incontinence materials for those over 65.¹⁷³ Those living closer to regions affected by active hostilities experience even greater hardships due to the direct impacts of the war. For older individuals with limited mobility, accessing bomb shelters is especially challenging, as many lack ramps and elevators.

Humanitarian aid has played a crucial role in supporting older people, with food kits from organizations providing essential items like rice, oil, and canned goods. However, access to aid is not universal, as many older individuals face mobility challenges or struggle with the technology needed to register for assistance. Lacking familiarity with digital tools, many find it difficult to access information how to apply for aid or locate support, further complicating their ability to navigate an already complex humanitarian system.¹⁷⁴ Nevertheless, local communities often play a crucial role in supporting older people by facilitating the collection of information and the distribution of aid from international organizations. Many participants noted that associations in their apartment buildings help coordinate and streamline the distribution of food and relief boxes, ensuring that assistance reaches those in need more efficiently.



This UN assistance, these boxes, are brought by a vehicle. The head [of the condominium], together with those who should receive them, give each resident a package based on this list.

FGD 27, with older women (60+), Kherson oblast



172 FGD 13 and 23, with older men (60+), Dnipropetrovsk and Mykolaiv oblasts

173 **CARE International. 2023.** *Rapid Gender Analysis: Ukraine.*

174 **REACH. 2024.** *Multi-Sector Needs Assessment: Gender, Age, and Disability Situation Overview.* (Sample: 13,322 households)

Economy and livelihoods

Access to jobs

While most older FGD participants reported being retired, their modest pensions, coupled with rising costs of basic necessities such as healthcare, hygiene products, utilities, and food, have led some to consider returning to work to supplement their income and cover essential expenses. However, with many older individuals facing chronic health concerns and reduced physical capabilities, they are often barred from certain physically demanding jobs. Tasks that require heavy lifting, prolonged standing, or exposure to harsh conditions can be particularly challenging, effectively limiting their employment opportunities in several industries.



There are those who are not allowed to do so [work] due to their health. Right? You won't go to work in a meat processing plant, a dairy, or a fish cannery

FGD 7, with older women (60+), Kharkiv oblast



Additionally, even when older individuals are qualified and willing to work, they often face age discrimination, with employers hesitant to hire them due to perceived limitations or concerns about long-term productivity. In general, 26 percent of Ukrainians believe that there is age discrimination in the Ukraine.¹⁷⁵ Finally, most of older people participants reported not receiving remittances from family members.

Community structure and meaningful participation

Information and knowledge management

Despite the efforts of humanitarian organizations to spread information about available assistance, many older individuals and those without digital access continue to struggle to stay informed. When it comes to receiving information on assistance, Dnipropetrovsk and Vinnytsia oblasts stand out for their localized communication practices: in Dnipropetrovsk oblast, housing cooperatives post announcements on building doors, fostering community-driven information sharing, while in Vinnytsia oblast, residents rely on in-person updates through city councils and church announcements. In fact, many participants expressed preferences for direct, accessible methods of communication. According to REACH, among older people-only households, 40 percent prefer face-to-face interactions,

175 **Kharkiv Institute of Social Research. 2025. Ageism in Ukraine: round discussion.**

while 32 percent favor phone calls.¹⁷⁶

In Lviv oblast, older FGD participants prefer to be contacted directly by phone, while in Kharkiv oblast, residents appreciate clear text message updates from territorial centres. In Dnipropetrovsk, Mykolaiv, and Zaporizhzhia oblasts, Viber is a popular platform for receiving information. However, older individuals would benefit from simpler, more direct methods such as phone calls or public announcements.¹⁷⁷ Participants in Vinnytsia oblast highlighted the importance of local government offices, city councils, and in-person communication as reliable sources.¹⁷⁸ In Sumy oblast, although Facebook and other social media platforms occasionally provide updates, there is a clear preference for more consistent and accessible channels to ensure broader community reach.¹⁷⁹ In addition, older women in Odessa oblast reported being hesitant to rely on online platforms to receive assistance, due to concerns about scams and fraud.¹⁸⁰

These challenges go beyond communication, directly impacting access to assistance. Older people, including those with disabilities, often face significant difficulties navigating digital registration processes. In the Mykolaiv oblast, older FGD participants reported facing difficulties completing the steps required to receive aid due to challenges using IBANs and providing geolocation information.

¹⁸¹ Online registration systems that operate on a first-come, first-served basis tend to favor individuals with fast internet connections and strong digital skills, enabling them to secure aid more quickly. In contrast, those using slower devices, dealing with poor connectivity, or lacking digital literacy are often unable to register on time and are consequently excluded. Addressing these barriers through alternative registration methods and simplified application procedures could ensure a more inclusive approach to humanitarian support.

Protection risks

Community safety

The sense of safety among older communities in Ukraine varies by oblast and is shaped by factors such as conflict, economic hardship, and limited access to healthcare. In Lviv, concerns include a rise in crime driven by financial struggles and occasional tensions related to language differences, particularly for older

176 **REACH. 2024.** *Multi-Sector Needs Assessment: Gender, Age, and Disability Situation Overview.* (Sample: 13,322 households)

177 FGD 12, 19, 23, with older men and women (60+), Dnipropetrovsk, Mykolaiv, and Zaporizhzhia oblasts

178 FGD 39, with older men (60+), Vinnytsia oblast

179 FGD 30, with older men (60+), Sumy oblast

180 FGD 21, with older women (60+), Odessa oblast

181 FGD 23, with older men (60+), Mykolaiv oblast

Russian speakers.¹⁸²

Across assessed oblasts, older individuals often struggle to access adequate healthcare, shelters, and distribution points due to mobility challenges and limited financial resources. These difficulties, compounded by loneliness, further heighten their sense of insecurity, especially during health emergencies. As a result, many older individuals report feeling neglected.



If you get to the hospital and you have no money, they won't even treat you, they won't even accept you. They won't even look in your direction.

FGD 21, with older women (60+), Odesa region



Roma community

Demographics and current humanitarian situation

Since the last census in Ukraine was conducted in 2001, and given the subsequent occupation of certain territories by Russia as well as widespread internal and external displacement, it is difficult to determine the exact number of Roma people currently living in the country. According to estimates, approximately 200,000 to 400,000 Roma people lived across Ukraine before the full-scale invasion. The largest Roma communities were in the Kharkiv, Odesa, and Zakarpattia oblasts.¹⁸³ Following the full-scale invasion in 2022, around 100,000 Roma people, predominantly women and children, were forcibly displaced, with approximately 50,000 leaving Ukraine to seek safety in other countries.¹⁸⁴

The Roma community in Ukraine is highly diverse, and despite differences in education, income, and place of residence, they continue to face the highest levels of prejudice and discrimination.¹⁸⁵ Ongoing difficulties with social integration increases their vulnerability, limiting access to essential services, legal protections, and economic opportunities, especially during wartime. This necessitates strong, targeted support from both governmental and non-governmental organizations to ensure inclusion, protection, and access to aid.

Provision and access to basic services and resources

182 FGD 36, with older men (60+), Lviv oblast

183 Minority rights group. 2020. *Roma in Ukraine*.

184 Голос Ромні. 2024. *Становище ромських громад під час війни в Україні*. (512 respondents)

185 KIIS. 2022. *Interethnic Prejudice in Ukraine*.

Compact Roma settlements are frequently established without legal authorization,¹⁸⁶ resulting in the absence of essential infrastructure such as centralized water supply, electricity, and heating systems. Even in areas where basic services are available, residents often report poor quality and limited reliability. During FGDs conducted in Marhanets (Dnipropetrovsk oblast) and Vysokopillia (Kherson oblast), Roma participants highlighted that the water in their homes was unsuitable for cooking or drinking, necessitating travel to obtain clean water or dependence on humanitarian aid.¹⁸⁷ Additionally, gas pipelines have been damaged due to ongoing military actions, and no repair efforts are currently underway. As a result, residents are compelled to use firewood for both heating and cooking purposes, which they are often forced to collect themselves – making this a physically demanding and time-consuming coping strategy.

Considering these challenges, the primary needs identified within Roma communities include firewood, access to clean water, hygiene kits (such as diapers and sanitary pads), essential medicines, children's clothing, and home repairs.



It is very difficult to live with the amount of money I have. I go to the pharmacy every month and throw money away. It was warm in the summer, but now I also have to buy or collect firewood. But my health doesn't allow me to go and collect firewood. I either buy food or firewood or pills for myself.

FGD 15, with Roma community, Dnipropetrovsk oblast



Ethnic discrimination significantly hinders Roma communities from accessing not only medical services but also a wide range of public services that require interaction with state institutions. Due to fears of discriminatory treatment, many Roma individuals actively avoid contact with governmental representatives. This reluctance is often compounded by the absence of essential personal identification documents, such as birth certificates and passports, which further restrict their ability to access basic services.

186 Compact Roma settlements are areas where a significant number of Roma people live in close proximity, often forming ethnically homogeneous communities.

187 FGD 15 and 28, with Roma community, Dnipropetrovsk and Kherson oblasts



The Roma population faces documentation issues. UNHCR works closely with Roma civil society to ensure that they have birth certificates, citizenship papers and other documentation. This is crucial because having documentation makes it harder to deny them their rights, including access to health care, financial support and other services

KII 20 with working group staff



The lack of proper documentation significantly restricts access to housing after forced displacement, making it difficult for Roma individuals to secure rental agreements. Beyond these bureaucratic barriers, discriminatory practices by landlords further restrict their housing options.¹⁸⁸

Provision and access to assistance

Some NGOs frequently exclude Roma people from aid distribution lists, often due to stereotypes that portray the Roma community as isolated, making it difficult for non-specialized organizations to establish contact. However, representatives from organizations actively working with Roma populations refute this notion, emphasizing that Roma individuals are present on digital platforms and accessible via phone, making it feasible for NGOs to provide support when committed to inclusive outreach.



You can get access, and anyone who wants to help can get access to the Roma community, you just have to do it and that's it.

KII 1 with an NGO representative working with Roma communities



Additionally, access to humanitarian assistance typically requires basic identification documents, such as a passport or a tax identification number. However, many Roma individuals lack these essential documents, which automatically excludes them from receiving aid. This barrier prevents them from accessing food distributions, financial support, medical aid, and other critical relief services. As a result, Roma communities remain disproportionately vulnerable

188 KII 14, INGO

in crisis situations, unable to benefit from humanitarian programs designed to provide assistance.¹⁸⁹

Economy and livelihoods

Access to jobs

Access to stable employment and fair wages remains an unmet need within Roma communities in Ukraine. According to a quantitative survey conducted by “Active Group” for the NGO “Golos Romni,” 62 percent of Roma respondents reported being entirely dissatisfied with the availability of jobs and decent wages.¹⁹⁰ A representative from an organization working with Roma communities estimated the employment rate among Roma people to be approximately 40 percent – slightly below the estimated national employment rate of 45 percent for individuals aged 15 and above in 2024.¹⁹¹

Employment is influenced by multiple structural barriers, including:

- **Ethnic discrimination:** Widespread stereotypes, such as the belief that Roma individuals are prone to theft, lead many employers to reject Roma job applicants.
- **Low literacy levels:** A significant portion of the Roma population lacks basic literacy skills, including reading and writing, which poses a barrier to employment opportunities.
- **Limited qualifications:** Due to low literacy rates, many Roma individuals have not completed formal education, including vocational or higher education, which limits their access to skilled labour markets.

As a result, Roma individuals are often restricted to low-skilled or seasonal employment. For example, many Roma women participate in agricultural work, such as harvesting, which is primarily available during spring and summer. However, during winter, rising heating costs, dwindling supplies of home-grown food, and a decline in job opportunities compound financial hardships, straining household resources.

Women in Roma communities are frequent recipients of various forms of humanitarian assistance, while men are more commonly engaged in informal side jobs to support their households.¹⁹² A distinction also exists between Roma individuals residing in compact settlements and those living separately in urban

189 KII 14, INGO

190 Голос Ромні. 2024. *Становище ромських громад під час війни в Україні*. (512 respondents)

191 KII 1, NGO staff working with Roma communities; World Bank. 2025. *Macro Poverty Outlook*.

192 Голос Ромні. 2024. *Становище ромських громад під час війни в Україні*. (512 respondents)

areas or suburban regions. For those living in compact settlements, income is often unstable and comes primarily from informal activities such as begging and fortune-telling. In contrast, Roma individuals living independently in cities or suburban areas more frequently report having stable employment and generally deny engagement in fortune telling as a source of income.¹⁹³

Community structure and meaningful participation

Information and knowledge management

Information about available humanitarian support within Roma communities is primarily disseminated through community leaders and direct personal communication with community members. Some respondents also indicated that they occasionally visit NGO offices to obtain information.¹⁹⁴ Additionally, details regarding assistance may be shared through messaging platforms such as Telegram and Viber. A representative from an NGO working with Roma communities emphasized that Viber is particularly widespread among Roma populations, underscoring the importance of utilizing this platform for effective outreach.¹⁹⁵

Decision-making processes

Decision-making regarding health, well-being, nutrition, and other forms of assistance within Roma communities is primarily centralized around the community leader, often referred to as the “block person.” This individual typically gathers information about the needs of community members and plays a key role in coordinating support.¹⁹⁶

Additionally, social workers or representatives from humanitarian organizations occasionally visit Roma settlements. Based on their observations during these visits, they assess the community’s needs and determine the type of assistance required.¹⁹⁷

Conflict dynamics

Social cohesion

It is important to note that opportunities for socialization outside the Roma community tend to be greater for men. Women, on the other hand, are more constrained by traditional norms and behavioral expectations, primarily interacting with family members or individuals closely connected to the family, which limits

193 Голос Ромні. 2024. *Становище ромських громад під час війни в Україні*. (512 respondents)

194 FGD 15 and 28, with Roma community, Dnipropetrovsk and Kherson oblasts

195 KII 1, NGO staff working with Roma communities

196 FGD 15, with Roma community, Dnipropetrovsk oblast

197 FGD 15, with Roma community, Dnipropetrovsk oblast

their social engagement beyond these circles.¹⁹⁸

To support the integration of Roma communities into Ukrainian society, NGOs should ensure that aid is provided in an inclusive manner. Efforts should focus on fostering integration by engaging Roma individuals alongside the general population.

For example, a representative from an organization working with Roma communities shared a successful practice where their Roma-led organization hosted a children's event open to both Roma and non-Roma children. Additionally, they organized a summer camp in the Carpathian Mountains with designated quotas for Roma children while ensuring participation was open to children from other backgrounds. Such inclusive approaches help break down barriers, encourage intercultural exchange, and support long-term social cohesion.¹⁹⁹

In many cases, women are expected to surrender their earnings to male family members, seek permission or accompaniment to leave the home, and refrain from sitting at the same table as men during meals. However, these practices are not commonly perceived by women as discriminatory. Instead, they are often viewed as cultural traditions, with some women finding comfort in adhering to the customs followed by their ancestors.²⁰⁰

Human rights violations

Roma individuals face systemic oppression and widespread discrimination across nearly all spheres of social life, including:

- **Education:** Instances where parents protest against their children studying alongside Roma children, as well as prejudiced treatment from teachers.
- **Healthcare:** Denial of medical services
- **Employment:** Refusal to hire Roma individuals due to discriminatory stereotypes.
- **Housing:** Rejection from landlords when attempting to rent apartments.
- **Public Perception:** Frequent exposure to hate speech targeting Roma communities.²⁰¹

Discrimination within Roma communities tends to disproportionately affect those with darker skin tones and black hair, as physical appearance often plays

198 Voice of Romni. 2024. Voice of Romni: Identity, gender aspects, and traditions

199 KII 1, NGO staff working with Roma community

200 Голос Ромні. 2024. *Становище ромських громад під час війни в Україні*. (512 respondents)

201 Ibid

a significant role in reinforcing ethnic stereotypes. Some respondents reported attempting to mitigate discrimination by altering their appearance, such as dyeing their hair, to avoid being perceived as Roma.



Those who look more like gypsies might be ‘attacked’. And if you are born with red hair or blond hair, how will they know that you are a gypsy? There are redheaded girls. They are not called gypsies. So, you have to dye your hair.

FGD 15, with Roma community, Dnipropetrovsk region



Protection risks

Gender-based violence

GBV remains prevalent within many Roma communities. As previously noted, a significant number of Roma women are unaware that they are experiencing systemic violence, as harmful practices are often normalized within the cultural framework. For instance, physical punishment is frequently perceived as an inherent aspect of Roma identity and tradition rather than a violation of rights. Additionally, social norms in many Roma communities discourage women from discussing family matters with outsiders, including other Roma. As a result, it is uncommon for Roma women to report GBV-related incidents to law enforcement authorities, further perpetuating cycles of violence and limiting access to protection and justice for survivors.²⁰²

People with Disabilities

Demographics and current humanitarian situation

Depending on the degree of health impairment, adults are assigned to one of three disability groups:²⁰³

- Group I Disability – individuals who have completely lost their ability to work and require constant care.
- Group II Disability – individuals who are capable of self-care but unable to work

202 Voice of Romni. 2024. Voice of Romni: Identity, gender aspects, and traditions

203 Resolution of the Cabinet of Ministers of Ukraine № 1317

under standard working conditions.

- Group III Disability – individuals who are able to work under light-duty conditions.

As of 2024, an estimated 3 million people in Ukraine are living with disabilities. This figure has increased significantly since the start of the full-scale invasion, primarily due to military actions causing debilitating injuries to soldiers and civilians alike. Prior to the war, around 13,000 individuals were diagnosed with a new disability annually. Since the full-scale invasion this number has risen dramatically to approximately 150,000 new cases per year.²⁰⁴ Additionally, the types of injuries have shifted, with a substantial increase in sensory impairments, such as hearing and vision loss.²⁰⁵

The humanitarian situation for people with disabilities and their families is dire. Over 2 million people with disabilities are in urgent need of assistance²⁰⁶, as the majority live below the poverty line. Humanitarian aid is a crucial lifeline.



...without those [humanitarian] organizations, without the humanitarian response that exists within the framework of various clusters, UN agencies, our people [people with disabilities] would not have survived

KII 10 with an NGO representative working with people with disabilities



Provision and access to basic services and resources

People with disabilities reported similar levels of service provision compared to people without disabilities. However, disruptions in the provision of basic services disproportionately affect them, exacerbating their vulnerabilities and limiting their access to essential resources. For example, individuals with limited mobility cannot obtain water without assistance during central water supply outages. Similarly, power cuts leave homebound people unable to leave their homes and immobilize those who rely on electric wheelchairs, which cannot be recharged.²⁰⁷

Access to facilities such as shelters, and hygiene rooms presents additional challenges. Many shelters remain physically inaccessible to people with disabilities

204 Ministry of Social Policy Ukraine. 13.06.24. Main news.

205 KII 10, NGO staff working with PWD

206 OCHA. 2023. Humanitarian needs and response plan Ukraine.

207 FGD 9 and 37, with PWDs, Kharkiv and Lviv oblasts

owing to lack of elevators and ramps, for example, and public buildings often lack hygiene rooms adapted to their needs, preventing independent use. While 34 percent of the general population say that bomb shelters are sufficient, 2 percent of persons with Group III disability status say they are sufficient, alongside 23 percent of persons with Group II status, and 22 percent of those with Group I status.²⁰⁸ Evacuation also remains a critical challenge, as it is often carried out by untrained volunteers who may lack the necessary skills and resources to safely assist people with disabilities. This heightens the risks for those affected, leaving them particularly vulnerable in high-risk situations where specialized support is essential.

Medical care is both the most essential and the most challenging service for people with disabilities.



We repeatedly hear from our colleagues and partners that the health of people with disabilities has significantly deteriorated due to the deterioration of access to medical supplies and medical care in general

KII 10 with an NGO representative working with people with disabilities



According to a UNDP report, somewhat or very efficient healthcare is reported by 42 percent of people with disabilities in regions affected by active hostilities, compared to 49 percent of people without disabilities in regions affected by active hostilities.²⁰⁹

Several key barriers hinder access to medical facilities for people with disabilities:²¹⁰

- **Physical inaccessibility:** Many facilities, such as X-ray rooms and gynaecological chairs, are not suitable for people with disabilities.
- **Staff shortages:** The deficit of medical professionals is particularly acute in areas close to the frontlines.
- **Medication accessibility:** People with disabilities often cannot afford necessary medications, many of which must be imported. Access to medicines is further complicated by logistical and pricing challenges due to ongoing

208 USAID and UNDP. 2023. *A Resilient Picture: Experiences of Persons with Disabilities in Ukraine*. (Sample: 549 respondents)

209 Ibid

210 KII 10, NGO staff working with PWDs; FGD 9 and 37, with PWDs, Kharkiv and Lviv oblast

military operations.

Overall, among people with disabilities, the most vulnerable in terms of access to resources are the bedridden and members of the first disability group.²¹¹

While education is not a direct concern for most adult people with disabilities interviewed, it poses significant challenges for children with disabilities, particularly in the context of war. With many having transitioned to distance learning, children with intellectual, mental, and sensory disabilities face substantial difficulties in accessing education, as remote formats often fail to accommodate their specific learning needs.



The first is access to education for children with disabilities, because in the conditions of war, many of them have switched to distance learning, and those with intellectual, mental, and sensory disabilities suffer greatly here. As well as access to education for our injured defenders, when they need to receive a new direction of education for their further activities

KII 10 with an NGO representative working with people with disabilities



Provision and access to assistance

87 percent of families with a member with a disability reported wanting aid in the future, while only 25 percent received assistance in the past three months.²¹² Two-thirds (66 percent) of people with disabilities use welfare payments frequently compared to 29 percent of people without disabilities. Additionally, 13 percent of people with disabilities use humanitarian aid frequently, compared to six percent of people without disabilities.²¹³

Despite ongoing aid efforts, participants identified several challenges in the delivery of assistance. Due to the overall decline in available support and the tightening of eligibility criteria, assistance is now primarily allocated to the people in the most vulnerable situations. As such, support for individuals with third-group disabilities has declined, leaving them in increasingly vulnerable conditions.

211 FGD 9 and 37, with PWDs, Kharkiv and Lviv oblasts

212 REACH. January 2024. Multi-Sector Needs Assessment: Gender, Age, and Disability Situation Overview REACH Initiative. (Sample: 13,322 households)

213 USAID and UNDP. 2023. *A Resilient Picture: Experiences of Persons with Disabilities in Ukraine*. (Sample: 549 respondents)

Beyond reduced support, logistical and structural barriers further limit access. In some cases, organizations purchase mobility aids without considering individual size requirements, increasing the risk of injury. Additionally, the limited number of social workers makes it difficult to provide home visits and properly assess the needs of those with mobility challenges, leaving many without essential support. Online registration processes also create barriers for older individuals and people with hearing impairments, effectively excluding them from accessing vital aid.²¹⁴

Moreover, organizations should tailor assistance to individual needs and actively engage with communities through door-to-door visits to assess needs and provide direct support. Additionally, alternative methods for delivering information about available assistance and enabling sign-ups should be implemented to ensure inclusivity. Participants also suggested updating international conventions and resolutions to better reflect local contexts, helping to address region-specific barriers and bridge the gap between international standards and practical implementation.²¹⁵

Economy and livelihoods

Access to jobs

The employment rate among people with disabilities over the age of 14 remains critically low, standing at approximately 20 percent.²¹⁶ For comparison, the 2024 forecast places this indicator at 45 percent for the general population.²¹⁷ This low rate is attributed to several factors, including persistent discrimination, stereotypes about people with disabilities' inability to perform job responsibilities or maintain long-term employment, and the lack of workplace adaptations for people with disabilities. By law, employers are subject to a four percent hiring quota for people with disabilities, but they typically prefer to pay a fine rather than hire people with disabilities.²¹⁸ Although the government offers compensation for workplace adaptations for people with disabilities, employers must initially cover the costs themselves. Additionally, reimbursement is not guaranteed and requires navigating complex bureaucratic procedures, including submitting documents in a specific format and providing detailed reports on all purchases. These procedures may deter employers from making necessary workplace adjustments.

214 KII 10, NGO staff working with PWDs; FGD 9 and 37, with PWDs, Kharkiv and Lviv oblasts

215 FGD 9 and 37, with PWD, Kharkiv and Lviv oblasts

216 The League of Strong. 2025. *The needs of persons with disabilities for livelihoods and access to the labor market in the context of war in Ukraine* (Sample: 1,019 respondents)

217 World Bank. 2025. *Macro Poverty Outlook*.

218 World Bank. 2024. *Ukraine - Third Rapid Damage and Needs Assessment (RDNA3) (Report No. P180174)*

Community structure and meaningful participation

Information and knowledge management

Awareness of available assistance among people with disabilities is generally low, hindered by fragmented and disorganized information. Even when information is available, it is often inaccessible to people with disabilities with intellectual disabilities, as well as people with hearing/visual impairments.

The primary channel for accessing information about assistance is online platforms, including messaging apps and social media groups, such as Telegram and Facebook. Other sources mentioned by respondents include street committees, homeowners' associations, territorial centres, social protection departments, and word of mouth. Occasionally, social centre representatives call individuals to inform them about available support.

Protection risks

Community safety

People with disabilities reported feeling unsafe, both physically and mentally, due to the ongoing war. This sentiment was also widely shared by the general population. People with disabilities described living in constant stress and tension, with the severity of these feelings often varying depending on their location. For instance, respondents from Kharkiv oblast highlighted these challenges more frequently. Some even reported turning to alcohol as a coping mechanism, though they acknowledged it provides little relief, while others found solace in housework or staying connected with friends and family.²¹⁹

For people with disabilities, their conditions often heighten their vulnerability – especially for those with severe disabilities (Groups I and II). Individuals unable to move independently are almost entirely reliant on others, even for basic tasks such as using their pension cards to purchase food or medicine.²²⁰ The situation is even more critical for individuals with psychosocial disabilities, who frequently face discrimination and social isolation as people avoid interacting with them.

219 FGD 9 and 37, with PWDs, Kharkiv and Lviv oblasts

220 KII 10, NGO staff working with PWD



We have a huge stigma around people with psychosocial disabilities. The perception of these people is that they are aggressive, dangerous to society, mentally ill people with intellectual disabilities. When in fact, if we go back to the jurisprudence, it is these people who suffer the most from crimes and offences because they cannot defend themselves.

KII 10 with an NGO representative working with people with disabilities



Human rights violations

30 percent of people with a Group I disability status report having been treated unfairly because of their health status or disability. This compares to 22 percent of people with Group II status and 20 percent of those with Group III status. Young people (aged 18 to 35) with disabilities report the highest levels of marginalization due to their health status.²²¹

Internally Displaced People

Demographics and current humanitarian situation

As of December 2024, there are an estimated 3,665,000 internally displaced people in Ukraine. The largest number are in Dnipropetrovska (14%) and Kharkiv (12%) Oblasts, while the highest proportion originate from Donetsk Oblast (28%). The most significant inter-oblast displacement movements have been from Donetsk Oblast to Dnipropetrovska Oblast (283,000 internally displaced people) and from Donetsk Oblast to Kyiv City (131,000 internally displaced people).²²² Unlike other conflict-affected populations who relocate far from their regions of origin, Ukrainian internally displaced people tend to move to adjacent oblasts. However, worsening security and access to basic services due to Russia's recent escalation in regions affected by active hostilities may drive more displacement westward.²²³

Females make up 59 percent of households made up of only displaced people, while males account for 41 percent. A majority (51 percent) are of working age,

221 USAID and UNDP. 2023. *A Resilient Picture: Experiences of Persons with Disabilities in Ukraine*. (Sample: 549 respondents)

222 IOM. 2025. *Ukraine Internal Displacement Report Round 19*

223 KII 13, cluster staff

with 23 percent being children and 26 percent older people.²²⁴ Younger internally displaced people are more likely to rely on employment rather than humanitarian aid, while older individuals and young mothers make up the majority of those seeking assistance at humanitarian hubs.²²⁵

Humanitarian efforts increasingly focus on regions affected by active hostilities, as well as collective sites and transit hubs where internally displaced people reside. Approximately 79,000 of Ukraine's internally displaced people with complex needs live in 1,800 sites across 23 oblasts, mostly in the west.²²⁶ Urban areas – while providing greater access to services – also expose displaced individuals, particularly adult men, to conscription risks.²²⁷

Provision and access to basic services and resources

Housing remains one of the most pressing challenges for internally displaced people, particularly in urban areas such as Kyiv city, where high rent prices and a lack of affordable accommodation exacerbate insecurity. Rental market instability, fluctuating prices, and precarious lease agreements further contribute to the issue. Landlords frequently raise rent, citing inflation or planned repairs, while some evict tenants to reclaim their property.²²⁸ As a result, just over one in four displaced households (26 percent) have moved to lower-quality dwellings, reflecting the challenges of sustaining adequate rented accommodation in protracted displacement.²²⁹ This issue is further compounded by winter-related vulnerabilities, as utility costs peak during colder months. Additionally, many internally displaced people struggle to secure new housing due to financial constraints and advocate for modular housing as a more sustainable solution.²³⁰

High utility and rental costs, combined with prolonged displacement, have contributed to severe financial vulnerabilities among internally displaced people. A significant portion report being unable to afford basic necessities such as power banks – especially critical due to increasing power outages – and essential medicines.²³¹ Access to housing depends on several factors, including the willingness of local authorities to engage with internally displaced people, the availability of temporary shelter, and community support for displaced populations. In areas where these conditions are met, internally displaced people

224 IOM. 2025. *Ukraine Internal Displacement Report Round 19*

225 KII 8, NGO staff working with IDP

226 OCHA. 2025. *Ukraine Humanitarian Needs and Response Plan 2025*.

227 KII 17, cluster staff

228 FGD 40, with IDPs, Vinnytsia oblast

229 IOM. 2025. *Ukraine Internal Displacement Report Round 19*

230 FGD 24, with IDPs, Mykolaiv oblast

231 IOM. 2025. *Ukraine Internal Displacement Report Round 19*

are more likely to stay longer, as housing poses less of a challenge.²³² However, discrimination in the rental market remains a significant barrier. Some landlords refuse to rent to internally displaced people, particularly families with children, older people, or those with disabilities, while others inflate rental prices upon learning that the prospective tenant is a displaced person. Negative perceptions of internally displaced people, often reinforced by isolated incidents of misbehavior, further contribute to this bias.²³³

For those who have lost property documents, proving ownership and reclaiming compensation for their homes is particularly difficult. With no dedicated government program to assist in restoring these documents, internally displaced people must navigate the standard judicial process, which is often lengthy and complex.²³⁴

Food insecurity is another challenge, especially for internally displaced people paying rent, with 26 percent reporting that they cannot afford food.²³⁵ Participants from modular settlements noted differences in available services, with some towns providing communal kitchens where food is served, while in others residents have to obtain and prepare their own meals, further straining their limited resources.²³⁶

Provision and access to assistance

The displaced people allowance (IDP allowance), introduced in March 2022 under Resolution 322, was amended in March 2024 to limit eligibility to specific groups due to rapid fund depletion. Initially, the allowance provided UAH 3,000 (USD 72) per month for people with disabilities and UAH 2,000 (USD 48) for other internally displaced people. In December 2023, new conditions were introduced, requiring unemployed internally displaced people to register at employment centres and discontinuing assistance for those who purchase assets exceeding UAH 100,000 (USD 2,402), or had bank accounts surpassing this amount.²³⁷

In March 2024, further amendments designated 10 vulnerable categories of internally displaced people (later expanded to 15) as eligible for six months of support, provided their household's monthly income did not exceed UAH 9,444 (USD 227).²³⁸ Certain groups of displaced people, including pensioners with low pensions, people with disabilities in groups I and II, orphans, and children deprived of parental care, received an automatic extension, while others were required

232 KII 8, NGO staff working with IDP

233 FGD 24 and 40, with IDPs, Mykolaiv and Vinnytsia oblasts

234 KII 8, NGO staff working with IDP

235 IOM. 2025. Ukraine Internal Displacement Report Round 19

236 FGD 33, with IDPs, Kyiv oblast

237 ReliefWeb. 2024. *Update on Changes to Payment of IDP Allowance: CMU Resolution 332.*

238 ReliefWeb. 2024. *Update on Changes to Payment of IDP Allowance: CMU Resolution 332.*

to reapply.²³⁹ According to IOM, these changes led to 39 percent of internally displaced people losing access to the allowance, including 36 percent in regions affected by active hostilities.²⁴⁰

The shift away from blanket assistance aims to foster economic self-sufficiency but requires significant investment in employment support, which currently exceeds the capacity of humanitarian actors. Development organizations are urged to enhance job placement initiatives, while humanitarian actors advocate for expanded livelihood support and improved referral mechanisms for people in vulnerable situations.²⁴¹ However, newly displaced individuals report similar challenges to those reported by earlier internally displaced people, particularly in securing essential items such as bed linens and blankets. As aid resources decline, meeting these basic needs has become increasingly difficult.²⁴²

Access to shelter remains a critical issue for a significant number of internally displaced people. The Shelter Program, which helped internally displaced people secure temporary housing and compensated homeowners hosting them, was terminated in September 2024 due to funding shortages. However, it is set to resume in January 2025 under revised conditions.²⁴³ Meanwhile, long-term housing solutions remain the government's responsibility, with expectations of compensation for destroyed homes or financial aid for permanent relocation.²⁴⁴

In response to the ongoing housing crisis, private foundations have launched construction projects for displaced individuals. However, internally displaced people residing in modular settlements report that neither they nor anyone in their communities have benefited from these projects.²⁴⁵

Beyond housing availability, administrators responsible for managing the settlement have highlighted pressing infrastructure needs, including paved pathways for wheelchair users and stroller access. Although these settlements fall under local government jurisdiction, internally displaced people report severe neglect of communal services, with essential maintenance, such as sewage disposal, occurring only twice over a 2.5-year period.²⁴⁶

Economy and livelihoods

239 ReliefWeb. 2024. *Update on Changes to Payment of IDP Allowance: CMU Resolution 332*.

240 IOM. 2024. *Ukraine IDP Allowance Update: Impact of the March 2024 Law Amendment on Ukraine's Internally Displaced*. (Sample: 40,00 respondents)

241 KII 7 and 12, Local government representatives and IO staff

242 FGD 33, with IDPs, Kyiv oblast

KII 8, NGO staff working with IDPs; FGD 14, with ID , Dnipropetrovsk oblast

243 KII 8, NGO staff working with IDP

244 KII 8, NGO staff working with IDP

245 FGD 33, with IDPs, Kyiv oblast

246 Ibid

Access to jobs

Internally displaced people face numerous challenges in securing stable employment and sustaining livelihoods. The widespread closure or relocation of businesses due to ongoing fighting has significantly reduced available job opportunities. While some internally displaced people have managed to find work with businesses that have relocated from conflict-affected areas,²⁴⁷ others have taken a more entrepreneurial approach. For example, a displaced woman from Mariupol established a bakery in Kyiv city, which not only provides employment to fellow internally displaced people but also contributes to local humanitarian efforts.²⁴⁸

However, many internally displaced people remain in unstable employment situations. Some are still technically employed by their pre-war employers but are not receiving wages, which prevents them from registering as unemployed and accessing official job-seeking services through the employment centres.²⁴⁹ This bureaucratic limbo further limits their ability to secure stable income and rebuild their lives.



Officially since they are technically employed, they cannot go and register at the employment centre. So, they do not get a wage, nor can they get hired elsewhere. People are hesitant to resign from their old jobs because they have hope to go back and if they do, they do not want to look for a new job. In theory they can resign and get registered with the employment centre and receive a voucher for a new profession

KII 10 with an NGO representative working with internally displaced people



Several employment programs have been established to support internally displaced people. For example, an organization focused on supporting displaced people in Ukraine has a Memorandum of Understanding (MoU) with some Employment Centres to engage internally displaced people in their operations, with compensation provided through a government-funded program. Internally displaced people participate in activities such as loading and unloading aid and contacting other internally displaced people to register and inform them about

247 FGD 24, with IDPs, Mykolaiv oblast

248 KII 8, NGO staff working with IDP

249 KII 8, NGO staff working with IDPs; FGD 24, with IDPs, Mykolaiv oblast

available assistance.²⁵⁰

Employment centres provide financial assistance for up to three months to support individuals in their job search.²⁵¹ They also contact registered job seekers to inform them of available opportunities; however, by the time individuals reach the Centre, many positions have already been filled.²⁵² Vocational training programs, some funded by the International Committee of the Red Cross and other international organizations, provide vouchers for truck driver training and other high-demand fields.²⁵³ Community-based initiatives, particularly those led by women's rights groups and local organizations, focus on empowering displaced women through skills training, contributing not only to employment but also to improved food security.²⁵⁴

Certain groups of internally displaced people face heightened employment difficulties. Older and adult internally displaced people who have lost employment records face challenges in proving their work history for pension eligibility. While electronic labor records have been maintained since 2000, archives from earlier years are incomplete, leaving some internally displaced people without the necessary documentation to claim their entitlements. Moreover, verifying employment is particularly difficult when the employer no longer exists or when individuals worked in areas that are now under Russian occupation.²⁵⁵

250 KII 8, NGO staff working with IDP

251 FGD 40, with IDPs, Vinnytsia oblast

252 FGD 24, with IDPs, Mykolaiv oblast

253 FGD 40, with IDPs, Vinnytsia oblast

254 KII 15, IO staff

255 KII 8, NGO staff working with IDPs; FGD 14 and 33, with IDPs, Dnipropetrovsk and Kyiv oblast



For those under 40, they can get more job opportunities through trainings, but above that there are limited opportunities. This is particularly an issue for people close to retirement age, because they cannot access jobs, nor can they access their pensions

FGD 40, with internally displaced people, Vinnytsia region



To cope with the lack of livelihoods, internally displaced people have switched to cheaper food, spent their savings, reduced utility usage, cut back on food consumption and essential medical expenses, and relied on humanitarian assistance.²⁵⁶ Overall, internally displaced people report using these coping mechanisms more frequently than non-displaced people. However, the greatest disparity is in dependence on humanitarian aid, with 51 percent of internally displaced people relying on it, compared to just 19 percent of non-displaced individuals.²⁵⁷

Community structure and meaningful participation

Decision-making processes

Internally displaced people believe that aid organizations determine the assistance they receive.²⁵⁸ Internally displaced people living in urban areas report less engagement with aid-providing organizations, whereas in villages, these organizations collaborate with local authorities—such as social services and village heads—who are more familiar with the needs of both the local community and internally displaced people.²⁵⁹

Some local administrators have established displaced people councils (IDP councils), with more than 1,000 operating across Ukraine.²⁶⁰ These councils serve in an advisory capacity, comprising equal representation of internally displaced people and local government officials involved in governance matters affecting internally displaced people, such as pension funds. However, these councils do not always function effectively.

256 FGD 24, with IDPs, Mykolaiv oblast; FGD 40, with IDPs, Vinnytsia oblast

257 IOM. 2025. *Ukraine Internal Displacement Report Round 19*. (Sample: 40,002 respondents)

258 FGD 14, with IDPs, Dnipropetrovsk oblast

259 FGD 24, with IDPs, Mykolaiv oblast

260 UNHCR. August 2024. *Displaced themselves, and now advocating for the rights and durable solutions for all internally displaced in communities across Ukraine*.



In theory these councils were made to collect the problems of internally displaced people on the ground and voice this issue to military administrations and city self-government bodies. In practice, the included internally displaced people were already involved in state power/local self-government employees, so they just did what they were told.

KII 8 with an NGO representative working with internally displaced people



Internally displaced people also report that while they receive their registration from one administrative body, aid is distributed through social services, which are often disorganized, frequently redirecting internally displaced people between different branches.



I wanted a voucher for a sanatorium. I went to social services. They gave me a paper. There are 30 offices. And I have to go around the city to find them and figure it out. You go there, and they say, “Why did you come here?” Then you go to another place, and they ask, “Why did you come here?”

FGD 24, with Internally displaced people, Mykolaiv oblast



For those living in temporary settlements, the manager of the settlement is the one that communicates aid needs to the relevant organizations and authorities.²⁶¹

Conflict dynamics

Social cohesion

While the difference in aid distribution sometimes fuels resentment between internally displaced people and local residents, it can also serve as a means of fostering solidarity. Some internally displaced people use the assistance they receive to support those in need within their communities.

²⁶¹ FGD 33, with IDPs, Kyiv oblast



Let's say I received humanitarian aid. I know someone in need—a woman who sells pies. She lives with her two sons, one struggling with addiction and the other with disabilities. She has high blood pressure but still gets up every day, bakes pies, and sells them to make ends meet. I know she's struggling too. So, if I have extra cereal or pasta, I share it with her. I bring her some pâté or stew, and in return, she gives me pies. It's a form of mutual aid. I also know that if I ever feel unwell, she—being a woman of faith—would come to help me faster than even the acquaintances who sheltered me but might not check in for weeks.

FGD 14, with internally displaced people, Dnipropetrovsk oblast



However, identity-based tensions – linked to political views, language, and sometimes religious beliefs – also affect relations between internally displaced people and host communities, especially for Internally displaced people from occupied areas such as Donetsk and Luhansk. In some regions, Russian-speaking internally displaced people reported feeling stigmatized or suspected of sympathizing with occupying forces, prompting them to adjust their speech or avoid openly disclosing their places of origin. Such differences occasionally result in social exclusion, impacting internally displaced people's integration, access to local services, or feelings of safety within their new communities.

While some internally displaced people build trusting relationships with their neighbors, others prefer to rely on fellow internally displaced people for support.²⁶² Within internally displaced people communities, strong social networks often form based on shared regions of origin. Initiatives like the 'I am Mariupol' network or annual Luhansk Day gatherings provide spaces for internally displaced people to connect and support one another.²⁶³

262 FGD 14 and 40, with IDPs, Dnipropetrovsk and Vinnytsia oblasts

263 KII 8, NGO staff working with IDP



Sievierodonetsk will unite with Sievierodonetsk. Mariupol residents have their own chat room called “I am Mariupol”. Crimeans have their own chat room and gather there. Luhansk residents gather once a year at the Pirogov Museum to celebrate the Day of Luhansk

KII 10 with an NGO representative working with internally displaced people



Informal support networks within host communities also exist, with residents occasionally offering food and household items to those in need. However, lingering resentment over aid disparities persists.²⁶⁴ Trust levels vary, with newer arrivals often struggling to integrate into their host communities.²⁶⁵ In some areas, shared work activities—such as seasonal fruit-picking—have helped bridge social divides and foster cooperation between internally displaced people and locals.²⁶⁶

Protection risks

Community safety

The sense of safety among internally displaced people is influenced by factors such as access to humanitarian aid, financial stability, and host community relations. Some feel protected due to family support and aid access, while others struggle with systemic challenges, including housing insecurity and economic instability.²⁶⁷ Rising rent prices, the threat of eviction, and perceptions of internally displaced people receiving preferential treatment contribute to social tensions and chronic stress.²⁶⁸

To improve safety and reduce social tensions, respondents suggest increasing financial support for housing, expanding temporary shelter options, and creating long-term housing programs for internally displaced people.²⁶⁹ Aid distribution processes should also be made more discreet, including options for direct home delivery and through Nova Poshta to reduce stigma.²⁷⁰ People in vulnerable situations, including older people, should receive targeted outreach, and aid

264 FGD 24 and 33, with IDPs, Mykolaiv and Kyiv oblasts

265 FGD 40, with IDPs, Vinnytsia oblast

266 Ibid

267 KII 8, NGO staff working with IDPs; FGD 14 and 40, with IDPs, Dnipropetrovsk and Vinnytsia oblast

268 FGD 40, with IDPs, Vinnytsia oblast

269 FGD 40, with IDPs, Vinnytsia oblast

270 Ibid

providers should enhance communication through direct phone calls rather than relying solely on digital platforms like Telegram.²⁷¹ Strengthening mental health support, such as integrating psychologists into aid teams, was also recommended to help address the emotional toll of displacement.²⁷²

LGBTQIA+ communities

Demographics and current humanitarian situation

Before the full-scale Russian invasion of Ukraine, LGBTQIA+ individuals were already among the country's most marginalized populations.²⁷³ The full-scale invasion has only exacerbated these vulnerabilities, especially for those who have become internally displaced.²⁷⁴ It is estimated that LGBTQIA+ individuals may comprise up to nine percent of Ukraine's internally displaced population, which would amount to over 300,000 people.²⁷⁵

Despite growing public support for LGBTQIA+ rights—evidenced by a sharp rise in favorable attitudes toward equal rights and civil unions—the humanitarian needs of queer individuals remain insufficiently addressed within broader displacement and relief efforts.²⁷⁶ Internally displaced LGBTQIA+ people face a unique convergence of risks, including increased exposure to violence, housing discrimination, limited access to trans-affirming healthcare, economic hardship, and challenges related to legal gender recognition and military restrictions.²⁷⁷

Ongoing support from LGBTQIA+ rights NGOs—particularly through shelter provision, legal aid, healthcare access, and emergency relief—has been vital. However, these efforts are under-resourced, and activists report growing burnout and capacity strains.²⁷⁸

Food and basic service delivery and assistance

Provision and access to basic services

LGBTQIA+ individuals in Ukraine continue to face significant discrimination that limits their access to essential services such as healthcare and shelter.

271 KII 8, NGO staff working with IDPs; FGD 40, with IDPs, Vinnytsia oblast

272 FGD 40, with IDPs, Vinnytsia oblast

273 Ilko Kucheriv Democratic Initiatives Foundation. 2020. *Human rights compliance assessment*.

274 Maryna Shevtsova. 2024. *Queering Displacement: The State of the Ukrainian LGBTQ+ Community During the Russian Full-Scale Invasion*.

275 Ipsos. 2023. *LGBT+ Pride Study 2023 Global Survey*.

276 Kyiv International Sociology Institute. 2023. *Perception of LGBT People and Their Rights in Ukraine: Results of a Telephone Survey*.

277 Gender in Detail. 2023. *Transgender people in Ukraine during a full-scale war*.

278 Maryna Shevtsova. 2024. *Queering Displacement: The State of the Ukrainian LGBTQ+ Community During the Russian Full-Scale Invasion*.

As the conflict exacerbates economic hardship and takes a toll on mental health, community members have reported a rise in GBV. Many have also been forcibly displaced after being evicted from their homes due to their sexual orientation or gender identity, further increasing the demand for shelter and humanitarian assistance.²⁷⁹ However, community members often avoid shelters established by authorities or NGOs due to fear of stigma and discrimination, as their identities are more likely to be visible in these spaces.²⁸⁰

As for medical care, the shortage of doctors not only limits general access to family doctors but also creates specific challenges for transgender individuals. For example, obtaining the F64.0 diagnosis – classified in the ICD-10 as “gender identity disorder in adolescence and adulthood” and required for legal gender recognition and access to hormone therapy –²⁸¹ has become increasingly difficult. Respondents highlighted that Territorial Recruitment Centres have an unwritten rule preventing transgender women from receiving this diagnosis, severely impacting their ability to update legal documents, find employment, and access medical treatment, including hormone therapy.²⁸² The financial burden of seeking care from private doctors further limits access to essential services. People living in occupied areas face even greater barriers in accessing essential medical services, including hormonal therapy for transgender individuals.

Provision and access to assistance

Humanitarian assistance primarily addresses basic needs such as cash transfers, food, and shelter, yet stigma remains a significant barrier for LGBTQIA+ individuals, who struggle to access housing, healthcare, and livelihood support.²⁸³ The ongoing demand for essentials like food, hygiene products, medical care, and psychological assistance highlights the instability many still face.²⁸⁴ A representative from an NGO working with the community emphasized that the ongoing demand for basic needs is concerning, as it indicates that many individuals have not been able to stabilize their living conditions.²⁸⁵

Members of the LGBTQIA+ community prefer to interact with organizations founded by LGBTQIA+ individuals or those with a history of working with the community. This is because LGBTQIA+ organizations know their needs and have developed specialized approaches to working with the community, prioritizing safety through online application forms, social media outreach, and targeted

279 KII 5, NGO staff working with PLHIV/AIDS & LGBTQIA+

280 KII 2 and 11, NGO staff working with LGBTQIA+ and UN HIV/AIDS staff

281 ICD-10. *F-64: Gender identity disorders*.

282 FGD 34, with LGBTQIA+, Kyiv oblast

283 OCHA. 2024. *Ukraine Humanitarian Needs and Response Plan 2024*.

284 FGD 34, with LGBTQIA+, Kyiv oblast; KII 2, NGO staff working with PLHIV/AIDS & LGBTQIA+

285 KII 2, NGO staff working with PLHIV/AIDS & LGBTQIA+

engagement in mobile apps. However, a significant portion of aid requests still comes through personal networks, where community members inform one another about available support.²⁸⁶

Members of the LGBTQIA+ community may choose to seek state assistance when it does not require physical presence, such as registering for displaced people allowances (IDP allowances). This reduces their barrier to accessing assistance because they are not required to disclose their sexual identity.



If I submit an application online, I can write anything. Applying depends on the honesty of the person. I could present myself as cisgender or anything else to receive aid. They won't check—you just fill out a Google form, and they process it and send the aid by mail. They won't verify who you really are.

FGD 34, with LGBTQIA+, Kyiv oblast



Although some organisations do not include LGBTQIA+ individuals, as a vulnerability criteria, it has been reported that including them as a separate category is not a viable solution, as many prefer not to disclose this information. However, recognizing LGBTQIA+ individuals as a specific vulnerability group could help ensure that their distinct protection needs – such as exposure to gender-based violence, legal discrimination, and social exclusion – are not overlooked in humanitarian response planning. At the same time, explicitly targeting LGBTQIA+ individuals as a vulnerability criterion may inadvertently increase their exposure to discrimination and security risks, particularly in hostile or conservative environments. This could discourage individuals from seeking aid and potentially compromise their safety.

Instead, assistance should be channelled through organizations already working with or founded by the LGBTQIA+ community, as these groups are better positioned to provide support in a discreet and context sensitive manner.²⁸⁷ For organizations already working with these communities, providing aid in occupied territories is particularly challenging. They cannot send physical humanitarian aid, nor can they transfer financial assistance, as bank cards and PayPal transactions are blocked.²⁸⁸ In regions affected by active hostilities, delivering aid is equally difficult due to ongoing shelling, which makes it dangerous to reach regional

286 Ibid

287 Ibid

288 FGD 34, with LGBTQIA+, Kyiv oblast

centres. Additionally, curfews further disrupt logistics for organizations operating in these areas.



The same applies to the duration of the curfew. Relatively speaking, this difference of one hour does not seem significant, but in reality, ground transport ceases operations after 8 p.m., with only the underground running until 10:30 p.m. Therefore, logistical barriers also exist in regions affected by active hostilities.

KII 2 with an NGO representative working with PLHIV/AIDS & LGBTQIA+



Organizations supporting the LGBTQIA+ community adapt their programming to ensure that beneficiaries across the country, including those in areas where they do not have a direct presence, can access assistance. For instance, organizations focused on these communities extend support to regions without established LGBTQIA+ organizations by providing food vouchers via email or offering remote IT courses in web design, hairdressing, cooking, and accounting.

They also operate specialized shelters for LGBTQIA+ individuals where residents receive clothing assistance, power banks, flashlights for power outages, and generators.²⁸⁹ They also distribute food certificates, hygiene items, and winter clothing to beneficiaries outside the shelters. Some also provide vocational training and retraining programs, which are in high demand. Mental health support is another key area of focus, with psychological counselling provided in many large cities and made accessible both remotely and in person.²⁹⁰ However, some LGBTQIA+ community members expressed dissatisfaction with the psychological counselling offered by aid organizations.²⁹¹

Economy and livelihoods

Access to jobs

Discrimination in employment remains a significant barrier for members of the LGBTQIA+ community in Ukraine. Similar to other cisgender men, gay and bisexual men often struggle to secure official employment unless they can present documentation confirming exemption from military service, leading many to opt

289 KII 2, NGO staff working with PLHIV/AIDS & LGBTQIA+; KII 5, NGO staff working with PLHIV/AIDS & LGBTQIA+

290 Ibid

291 FGD 34, with LGBTQIA+, Kyiv oblast

for informal or remote work instead. Transgender individuals face additional obstacles, as many have outdated documents due to the difficulty of obtaining the F64.0 diagnosis required for legal gender recognition.

While Ukraine’s Labor Code officially bans discrimination based on sexual orientation and gender identity, there are no real enforcement mechanisms. Employers do not openly reject candidates for these reasons, since it is illegal, but often justify their decisions by claiming the person lacks experience.²⁹²



A regulation of non-discrimination based on sexual orientation and gender identity exists formally in the Labor Code of Ukraine. Unfortunately, there are no real and substantial mechanisms for the protection of LGBTQIA+ people in cases of discrimination at the workplace.

KII 5 with an NGO representative working with PLHIV/AIDS & LGBTQIA+ communities



Despite these challenges, the service industry—including public catering, sales, and real estate rental—tends to be the most accepting sector for LGBTQIA+ individuals. However, not all members of the community work in this field ²⁹³

Community structure and meaningful participation

Information, knowledge management, and community feedback

To improve their chances of receiving aid, individuals often register with multiple organizations. In urban areas, registration is primarily conducted online; however, phishing and spam links have become more common, so individuals need to be more cautious.²⁹⁴

Despite strong internal networks, awareness of available aid remains limited within the LGBTQIA+ community. While close-knit groups facilitate support, they also restrict the spread of information beyond immediate circles. To address this gap, a structured referral system and greater operational sensitivity among NGOs is needed. Experienced organizations working with these communities are working to improve access by training social workers from other organizations on inclusion, aiming to enhance their understanding of LGBTQIA+ needs and improve their

292 Ibid
293 KII 2, NGO staff working with PLHIV/AIDS & LGBTQIA+
294 Ibid

service provision.²⁹⁵ However, many LGBTQIA+ organizations lack the resources necessary to scale these efforts effective .

Concerns about privacy and confidentiality further limit access to assistance. Many LGBTQIA+ individuals are reluctant to share personal or passport information due to fears of being outed, particularly within their families and communities. As such, individuals prefer to provide the minimum information required to receive assistance and may even opt out of aid if disclosure requirements are too extensive.²⁹⁶



For many LGBTQIA+ people, it is an issue provide their personal or passport data or provide more detail about their families. Since LGBTQIA+ people may hide this information from their families. It means the members of their families may not know about their sexual orientation or gender identity. This applies both to relations between children and parents, husband and wife.

KII 2 with an NGO representative working with PLHIV/AIDS & LGBTQIA+



Decision making

While LGBTQIA+ organizations strive to involve community members in decision-making, they recognize that gaps persist.²⁹⁷ There is a need for humanitarian organizations and aid providers to establish clear, actionable inclusion policies rather than relying on broad commitments that lack implementation.²⁹⁸ State social services, in particular, struggle with a lack of structured policies and frameworks to effectively support the LGBTQIA+ community. Although non-discrimination regulations exist, there is little practical guidance on how to enforce them.

Conflict dynamics

Social cohesion

Many LGBTQIA+ individuals avoid participating in broader social processes out of fear that their identities will be revealed, leading to stigma and discrimination.²⁹⁹ Some organizations engaging directly with these communities work to strengthen social cohesion and create safer spaces.

295 Ibid
296 Ibid
297 Ibid
298 Ibid
299 Ibid

One way to assess how well the LGBTQIA+ community is integrated into society is through reactions to Pride events. While these events serve as a visible measure of acceptance, they also attract hostility, especially in places like Kharkiv, where LGBTQIA+ gatherings frequently lead to online hate speech and, in some cases, hate-motivated crimes.³⁰⁰



Stigmatization from the outside, and violations of rights, aggression, attacks—unfortunately from other people, community representatives, right-wing radicals, and so on. Unfortunately, there is also physical violence in this case.

KII 5 with an NGO representative working with PLHIV/AIDS & LGBTQIA+ communities



To overcome these issues, organizations working with these communities actively work to strengthen social cohesion through various initiatives such as bringing together local LGBTQIA+ representatives and internally displaced people to foster stronger community connections and running community centers.³⁰¹

Protection risks

Community safety

For the LGBTQIA+ community, safety concerns go beyond the broader risks of war. Some members feel comfortable expressing their identities, while others fear harassment and violence. Transgender individuals often experience verbal abuse, with strangers yelling at them on the street, asking intrusive questions like, “Are you a boy or a girl?”. In some cases, this escalates into physical assault, with men more likely to be violent and women, often resorting to public shaming. Simply displaying LGBTQIA+ symbols can also make individuals a target for aggression.³⁰²

Despite these threats, many community members avoid reporting incidents to the police, fearing discrimination or retaliation. Previous negative experiences have made them distrustful of law enforcement.

300 KII 2, NGO staff working with PLHIV/AIDS & LGBTQIA+

301 Ibid

302 FGD 34, with LGBTQIA+, Kyiv oblast



Turning to law enforcement agencies is, in general, at the level of fantasy, due to having either a previous negative experience or actually fearing that they will have a negative experience disclosing their sexual orientation or gender identity.

KII 2 with an NGO representative working with PLHIV/AIDS & LGBTQIA+



Beyond public harassment, LGBTQIA+ individuals face serious risks within their own homes. If their sexual orientation or gender identity is revealed, they may experience abuse, be forced out of their homes, or be cut off financial . The full-scale invasion has made these situations worse, leaving many with few alternatives. In some cases, individuals remain in abusive relationships simply because they have nowhere else to go.

For gay, bisexual, and transgender men, military service presents another layer of risk. During evaluations by the medical commission for military conscriptions, their sexual orientation or gender identity can be exposed, which may expose them to discrimination.

While humanitarian aid is meant to support people in vulnerable situations, it can sometimes do more harm than good – particularly when services are provided in ways that reinforce stigma. This is a particular concern in medical settings, where discrimination can prevent people from accessing essential care.



If they are not inclusive of LGBTQIA+ people—if, conventionally speaking, a gynaecologist tells a lesbian experiencing a sexual health problem that it's because she does not have sex with men—it will obviously hurt more than help. Or if we are talking about services where, during their provision, for example, the sexual orientation or gender identity of a person was revealed. This can be a situation in shelters when humanitarian aid is provided.

KII 2 with an NGO representative working with PLHIV/AIDS & LGBTQIA+



When transgender people go to a medical facility, they might announce their name or even shout it out while they're in the hallway. There's a transgender man standing there, and they call out, 'Yana! Come in,' for example. That's already bullying. This happens frequently in medical institutions.

KII 2 with an NGO representative working with PLHIV/AIDS & LGBTQIA+



These cases underscore the importance of ensuring that LGBTQIA+ individuals have access to medical facilities and personnel who provide inclusive and supportive care.

To prevent aid distributions from putting recipients at risk, organizations working with LGBTQIA+ communities have strict security measures in place. They also carefully screen individuals before providing them with distribution addresses to ensure beneficiaries' safety.



If a person comes to us for the first time, we provide an address where people can come only if we have checked them to see if they are on the blacklist, or we also look at their social media. If we see that a person just created an Instagram and Facebook page yesterday without photos, then he or she could very well be a fake person. Unfortunately, there are different organizations, including right-wing radical ones. It is better for us to refuse such a person and not let them in. So, security is also very important.

KII 5 with an NGO representative working with PLHIV/AIDS & LGBTQIA+ communities



In addition to these precautions, anonymous communication, including interactions with accounts that have unclear avatars or nicknames, is accommodated to build trust and ensure accessibility. Furthermore, services are scheduled at times that align with clients' safety concerns and work schedules, including evenings and late hours. This flexibility reflects an understanding of the unique vulnerabilities and realities faced by the LGBTQIA+ community.³⁰³ LGBTQIA+ focused organizations also prioritize minimizing exposure to risk by offering as many services remote as possible. This includes psychological support, legal assistance, and peer-to-peer advice, as well as electronic food and hygiene certificates³⁰⁴

To help address challenges related to the feeling of insecurity, organizations working with LGBTQIA+ individuals prioritize creating safe spaces where community members can access support without fear of discrimination or hostility. Community centres operate on principles of equality, safety, and inclusion, ensuring an environment free from stigma. They take a sex-positive approach, encouraging open discussions about LGBTQIA+ needs and experiences without judgment. Events and services are tailored to the specific needs of the community, making them both relevant and inclusive. Furthermore, to enhance this group's sense of safety they also hold webinars on how to behave if they were to be confronted by the Territorial Centres of Recruitment and how to act in different situations.³⁰⁵

303 KII 2, NGO staff working with PLHIV/AIDS & LGBTQIA+

304 KII 5, NGO staff working with PLHIV/AIDS & LGBTQIA+

305 Ibid



It is mandatory, because there are different situations with them, and you need to understand how to behave, where to go, and when it's a situation where your rights are definitely being violated 100%.

KII 5 with an NGO representative working with PLHIV/AIDS & LGBTQIA+ communities



People Living with HIV/AIDS

Demographics and Current Humanitarian Situation

According to the Ukrainian Ministry of Health, the largest group affected by HIV/AIDS in Ukraine consists of people who use drugs, followed by men who have sex with men (MSM), LGBTQIA+, individuals, sex workers, prisoners, and former prisoners.³⁰⁶ Partners of people who inject drugs are also considered a key affected subgroup.³⁰⁷

In recent years, Ukraine has made significant strides in combating HIV/AIDS, with a 47 percent decrease in new HIV infections and an 81 percent reduction in AIDS-related mortality prior to the Russian invasion.³⁰⁸ As of 2023, 158,803 people live with HIV/AIDS.³⁰⁹ Despite disruptions caused by the war, the country has managed to resume its HIV/AIDS response, with 118,348 patients receiving antiretroviral therapy (ART) by the end of 2023 – only slightly below pre-war levels.³¹⁰ Encouragingly, the first nine months of 2023 saw a 40.8 percent increase in HIV testing, indicating continued efforts to maintain access to essential healthcare services.³¹¹

However, the war in Ukraine has disrupted healthcare systems, posing severe challenges for people living with HIV/AIDS (PLHIV). The displacement of healthcare professionals specializing in HIV treatment has also strained service provision and made it more difficult for organizations to deliver consistent support.³¹²

306 Ibid

307 Ibid

308 UNAIDS. 2024. UNAIDS response to the crisis in Ukraine.

309 WHO. 2023. *On the frontline of the fight against HIV: Ukraine's resilience and WHO's support*. UNAIDS. 2024. *UNAIDS response to the crisis in Ukraine*.

310 UNAIDS. 2024. UNAIDS response to the crisis in Ukraine.

311 Ibid

312 Cox. 2024. *Ukraine keeps up the fight against HIV while fighting a war*.

Provision and access to basic services and resources

The escalation of the invasion has affected PLHIV and people who inject drugs (PWID) differently depending on their social status. These vulnerabilities are further compounded in displacement, as the disruption of social networks and barriers to services in host communities increase their exposure to risk and further limit their access to support.³¹³

Among the most pressing issues faced by this group are access to healthcare and shelter.³¹⁴ While some shelters offer specialized support for PLHIV, access remains limited. Discrimination in displacement settings, along with limited access to traditional support networks among displaced and host communities, continues to deter individuals from seeking help.³¹⁵

The shortage of doctors has reduced access to family doctors, leading some to rely on private care, where high costs can further restrict access to essential services. Additionally, healthcare professionals with expertise in managing HIV have left the country, which creates challenges for organizations working with these populations, as it reduces the availability of specialized care and limits referral options.³¹⁶

For PLHIV, concerns over confidentiality of medical records persist, with reports of sensitive health statuses being disclosed publicly. Respondents also highlighted long waiting times and bureaucratic obstacles, particularly for oncology patients. Many noted that patients often have to advocate for themselves to receive proper documentation and treatment.³¹⁷

In occupied areas, people face severe barriers to accessing essential medical care. The conflict has also reportedly contributed to a rise in opioid injections due to stress in regions heavily affected by hostilities, further increasing the risk of HIV transmission.³¹⁸

Provision and access to assistance

The Ukrainian Public Health Centre, in collaboration with NGOs, has established minimum standards of support for PLHIV. These standards cover medical, social, and psychosocial services but remain basic, with considerable room for improvement to ensure comprehensive and sustainable care.³¹⁹ As the war in

313 OCHA. 2024. *Ukraine Humanitarian Needs and Response Plan 2024*.

314 OCHA. 2024. *Ukraine Humanitarian Needs and Response Plan 2024*.

315 UNAIDS. 2024. *UNAIDS response to the crisis in Ukraine*.

316 Cox. September 2024. *Ukraine keeps up the fight against HIV while fighting a war*.

317 FGD 25, with PLHIV/AIDS, Mykolaiv oblast

318 Cox. September 2024. *Ukraine keeps up the fight against HIV while fighting a war*.

319 KII 11, IO staff

Ukraine continues, strengthening HIV/AIDS programs and ensuring inclusive humanitarian responses that address the specific challenges faced by PLHIV in Ukraine is crucial.

PLHIV, MSM, and PWID primarily access assistance through NGOs that specialize in supporting them. They tend to rely on dedicated organizations and are unlikely to seek help from other NGOs, even when relevant humanitarian programs are available in their area. Familiarity with an NGO plays a crucial role in whether individuals seek or remain engaged in support programs. If an organization has not previously worked with HIV-positive individuals, potential beneficiaries may fear that accessing its services could expose their status, leading them to avoid assistance altogether.³²⁰

Economy and livelihoods

Similar to the experiences of LGBTQIA+ community members, many HIV-positive individuals fear disclosing their status in the workplace due to concerns about negative consequences or dismissal.³²¹

Community structure and meaningful participation

Similar to LGBTQIA+ members, concerns about privacy and confidentiality limit access to assistance. However, for PLHIV, access to essential medical treatments such as ART therapy, HIV treatment, and pre-contact HIV prevention requires passport data, as these services are provided through medical facilities.³²² This makes it especially important for medical centers to be trained and specialized in working with PLHIV, to avoid stigma or insensitivity in service provision. When healthcare providers are not adequately trained, individuals may feel unsafe or disrespected, which can lead them to avoid seeking care altogether.

Protection risks

Community safety

PLHIV face similar challenges to those experienced by members of the LGBTQIA+ community in medical settings, where stigma and a lack of understanding from healthcare providers can discourage them from seeking care. Just as with LGBTQIA+ individuals, access to inclusive, confidential, and respectful treatment is crucial. This highlights the importance of working with specialized organizations that understand this group's specific needs and vulnerabilities.

320 KII 11, IO staff; FGD 25, with PLHIV/AIDS, Mykolaiv oblast

321 FulcrumUA. 2024. *(Non)discrimination of LGBTQIA+ people in the workplace and inclusive labor market results of a national survey of the LGBTQIA+ community.*

322 KII 5, NGO staff working with PLHIV/AIDS & LGBTQIA+

HIV-positive individuals serving in the military face also face challenges in accessing HIV-related medical services, particularly antiretroviral therapy.³²³ Stigma surrounding HIV status poses a barrier for service members needing treatment, as many are afraid to disclose their status or seek medical care due to fears of being identified. In other cases, fear of being mobilized has led some to stop treatment altogether in an attempt to alter their medical classification and avoid conscription.³²⁴

323 KII 2, NGO staff working with PLHIV/AIDS & GBTQIA+; KII 5, NGO staff working with PLHIV/AIDS & LGBTQIA+

324 KII 5, NGO staff working with PLHIV/AIDS & GBTQIA+



Conclusion & Recommendations

CONCLUSION & RECOMMENDATIONS

As Ukraine enters its fifth year of the full-scale invasion, the conflict continues to impact key infrastructure, essential services, and the well-being of its people. Mass displacement, psychological distress, and shifts in international aid priorities further complicate the humanitarian response. In this dynamic context, a comprehensive understanding of conditions on the ground, including the diverse struggles and needs of individuals, particularly those in hard-to-reach communities, remains crucial for informing humanitarian efforts.

The I-CARA contributes to this understanding by offering a detailed picture of the humanitarian landscape in Ukraine. It highlights disparities in access to services, protection risks, and the diverse needs of women, men, and people in vulnerable situations. The analysis of the threat's vulnerabilities and coping mechanisms across different groups provides a foundation to design more effective and inclusive humanitarian programming. These insights are intended to support inclusive, context-sensitive programming, improving aid delivery, strengthening inclusion and accountability, and addressing both immediate needs and longer-term resilience.

The following summary reflects the perspectives and experiences shared by participants in focus group discussions and key informant interviews conducted as part of the I-CARA assessment. These insights do not constitute an evaluation of the work of WFP or any other humanitarian organization. They are intended to inform understanding of the humanitarian context in Ukraine and should be interpreted as indicative of community perceptions rather than definitive assessments of institutional performance.

Improving access to humanitarian aid

Access to humanitarian aid remains a critical concern across Ukraine, particularly in frontline and rural areas. While in-kind assistance continues to play a vital role where markets are disrupted, many affected individuals face significant barriers to accessing aid. These include long curfews, damaged infrastructure, limited public transportation, and digital literacy challenges. Vulnerable groups, such as older people, persons with disabilities, and Roma communities, are disproportionately affected, often lacking the documentation or technological means required to register for assistance. Older persons reported discontinuing medical treatments due to unaffordable costs, while those with mobility challenges rely heavily on local community support to access essentials like water and food. Roma communities living in compact settlements face acute challenges in accessing water, gas, and shelter, often resorting to collecting firewood for heating due to inadequate infrastructure and lack of formal service provision.

Cash assistance is widely preferred for its flexibility, enabling recipients to prioritize urgent needs like food, rent, and medicine. However, logistical constraints and eligibility criteria can limit its reach. Community members consistently emphasized the importance of maintaining both cash and in-kind modalities to ensure inclusive coverage, especially in high-risk and underserved areas.

Enhancing accountability and communication

Humanitarian communication efforts in Ukraine utilize a broad mix of digital and traditional channels, including social media, SMS, radio, and billboards. Despite these efforts, many respondents reported confusion regarding eligibility criteria, registration procedures, and aid availability. This was especially true for individuals with limited digital access or literacy. Older persons and individuals with disabilities, particularly those with hearing impairments or outdated devices, face compounded challenges in navigating digital platforms. This digital exclusion not only limits access to information but also hinders registration for aid and feedback mechanisms.

Direct communication, via phone calls, hotlines, and face-to-face interactions, was consistently preferred, particularly in rural and frontline communities. These methods were seen as more reliable and accessible, helping to build trust and reduce misunderstandings.

Feedback mechanisms are in place but vary in effectiveness. While platforms like Viber and Telegram are commonly used, concerns about anonymity and responsiveness persist. Many participants expressed a desire for more inclusive and transparent processes, noting that unclear targeting and perceived favoritism can undermine community trust. There was also a strong call for more dignified aid delivery, such as allowing recipients to choose items or reducing the visibility of aid collection.

Adopting inclusive and conflict-sensitive targeting

The assessment revealed that perceptions of fairness in aid distribution are closely tied to transparency and community involvement. While targeting frameworks are informed by needs assessments and government coordination, many community members felt excluded from decision-making processes. This sense of exclusion was particularly pronounced among marginalized groups, including Roma communities, LGBTQIA+ individuals, and people with disabilities.

Documentation requirements and digital registration systems were frequently cited as barriers, especially for displaced individuals and those lacking formal identification. Participants emphasized the importance of flexible targeting approaches that account for diverse vulnerabilities, including those not easily

captured by standard criteria. For example, older persons willing and able to work reported experiencing age discrimination, while Roma individuals described altering their appearance to avoid ethnic profiling. These insights highlight the need for more nuanced vulnerability assessments that extend beyond formal documentation and digital registration systems.

Efforts to improve inclusion, such as outreach through community leaders and tailored communication, were appreciated but not uniformly experienced. The findings suggest that continued attention to inclusive targeting is essential to ensure that assistance reaches those most in need, while minimizing social tensions and perceptions of inequity.

Safeguarding

Safety and dignity in aid delivery emerged as key concerns, particularly in regions affected by active hostilities. Overcrowded distribution points, long wait times, and exposure to security risks, such as shelling, were cited as challenges. Mitigation measures, including scheduled appointments, door-to-door deliveries, and discreet distribution methods, were seen as effective in enhancing safety and reducing stigma.

Protection risks vary across groups. For example, LGBTQIA+ individuals and people living with HIV/AIDS reported avoiding public shelters due to fears of discrimination or identity exposure. Similarly, Roma women described cultural norms that limit their mobility and access to services. These insights underscore the importance of context-sensitive approaches that prioritize safety, privacy, and dignity, particularly for groups facing compounded risks due to discrimination, mobility limitations, or social stigma.

Expanding livelihood opportunities

The war has significantly disrupted livelihoods, with widespread unemployment and economic instability affecting both displaced and host communities. While some industries have relocated to safer areas, job opportunities remain limited, especially in rural regions and among older adults, women with caregiving responsibilities, and people with disabilities.

Many respondents reported relying on informal work, remittances, and humanitarian assistance to meet basic needs. Vocational training and reskilling programs were identified as valuable pathways to economic recovery, particularly for displaced individuals and women entering new roles as household breadwinners.

However, structural barriers, such as age discrimination, lack of childcare, and

limited access to formal education, continue to hinder employment. Participants emphasized the need for sustained support to rebuild livelihoods and reduce dependence on aid. Roma and LGBTQIA+ individuals, as well as people living with HIV, expressed a preference for working with specialized organizations founded by or closely connected to their communities, citing greater understanding and tailored support.

Strengthening protection and inclusion

Protection risks are shaped by both conflict dynamics and social exclusion. Displaced individuals, LGBTQIA+ communities, Roma people, and persons with disabilities face heightened vulnerabilities, including discrimination, limited access to services, and exposure to violence. These risks are compounded by stigma, limited availability of specialized support, and gaps in legal protections.

Participants highlighted the importance of inclusive service provision, noting that assistance delivered through trusted organizations, particularly those led by or working closely with marginalized groups, was more likely to be accessed and valued. Ensuring confidentiality, cultural sensitivity, and tailored support were seen as essential to fostering inclusion and reducing harm.

Fostering community cohesion and social integration

Social cohesion in Ukraine has evolved throughout the conflict, with initial tensions between displaced and host communities gradually giving way to solidarity and mutual support. Informal networks, such as neighbors sharing food or helping with daily tasks, play a vital role in sustaining resilience.

However, perceived inequities in aid distribution and differences in language, political views, or cultural norms can still fuel mistrust. Displaced individuals reported both positive and negative experiences integrating into new communities, with some forming strong bonds and others facing exclusion or suspicion. Roma communities reported persistent ethnic discrimination, including limited access to public services and social exclusion. Some individuals even described altering their appearance to avoid being identified as Roma, underscoring the depth of stigma and barriers to integration. Persons with disabilities and older adults also highlighted their reliance on informal networks for daily support, an essential lifeline that may not be sustainable without formal inclusion mechanisms.

Efforts to promote inclusive community engagement, such as joint activities, shared work initiatives, and culturally sensitive programming, were seen as effective in bridging divides and strengthening the social fabric. These findings highlight the importance of fostering trust and collaboration at the local level to

support long-term recovery and integration.

Evolving Gender and Social Norms

The conflict has significantly reshaped gender roles and social expectations across Ukraine. Women have increasingly assumed roles as primary breadwinners and caregivers, often balancing informal work with household responsibilities in the absence of male family members. Despite the increasing burden of responsibilities, many women reported feeling unprepared for these new roles, citing limited opportunities to acquire new professions or necessary qualifications. Inadequate childcare support further compounds these challenges, particularly for internally displaced women, who face additional barriers to securing employment. Meanwhile, men face heightened protection risks due to conscription, which limits their mobility and access to formal employment. The requirement for military documentation to engage in formal employment discourages many from registering for assistance or seeking stable jobs, contributing to a sense of vulnerability. Although men did not explicitly describe themselves as isolated, the fear of conscription and avoidance of formal systems may lead to social and economic marginalization over time. These shifts have introduced new vulnerabilities and coping strategies, while simultaneously challenging traditional gender and social norms. Participants emphasized the need for continued support that acknowledges these evolving dynamics and promotes equitable access to opportunities and services.

Localization and the role of national and local actors

As humanitarian operations in Ukraine increasingly transition from emergency relief to resilience-focused programming, the role of national and local actors has become more prominent. The decentralization of aid delivery, through oblast, raion, and hromada-level structures, has enabled more context-specific responses, but also introduced challenges related to capacity, coordination, and consistency. While local NGOs and authorities are often well-positioned to identify needs and deliver assistance, participants noted that their ability to lead and influence strategic decisions remains limited. Strengthening collaboration between international and local actors, while ensuring that cross-cutting priorities such as inclusion, safeguarding, and accountability are maintained, will be essential to sustaining effective humanitarian action in the evolving Ukrainian context.



Annexes

ANNEXES

Annex A. Analytical Framework

Component	Subcomponent	Indicator	Data collection mode		
			DR	KII	FGD
Demo-graphics	Regular and hard-to reach audiences	- Demographic composition and size of each group - Age/gender distribution	✓	✓	
	Provision and access to basic services and resources (e.g., shelter, education, health, water, electricity, gas, transportation, community assets, financial services, land ownership etc.)	- Provision and access to basic services and resources (e.g., shelter, education, health, water, electricity, gas, transportation, community assets, financial services, land ownership etc.) Capacity of the different communities to meet family needs (housing/shelter, household items, water, hygiene, and winter) - Basic service needs/preferences of affected people - Groups of people facing challenges in accessing resources - Reasons for the exclusion of certain groups of people from accessing resources - Existence of community rules and laws that prevent certain people from accessing resources - Differences in access and participation in the design of basic service delivery	✓	✓	✓
Service delivery and assistance	Food security and coping strategies	- Profile of food-insecure or vulnerable groups of people - Causes of food insecurity - Dynamics of the development of the food insecurity situation - Coping strategies for addressing food insecurity	✓	✓	✓
	Provision and access to assistance	- Mechanisms for receiving assistance (Potential) obstacles or problems faced accessing assistance or participating in humanitarian activities across the different communities - Identify groups of people excluded from receiving assistance and the drivers of their exclusion - Mitigation measures to ensure equal access to assistance - Instances of favor-based humanitarian assistance distributions - Differences in access and participation in the design of assistance provided	✓	✓	✓

Component	Subcomponent	Indicator	Data collection mode		
			DR	KII	FGD
Economy and livelihoods	Access to jobs	• Employment status, sector of work, type of work (full/part-time) • Barriers to employment	✓	✓	✓
	Human, social, natural, physical, and financial capital	• Dependence on social network • Livelihood coping strategies	✓	✓	✓
Community structure and meaningful participation	Information and knowledge management	• Awareness of sources of information about different forms of assistance	✓	✓	✓
	Decision-making processes	• Health and well-being decision makers within the different communities		✓	✓
	Community feedback and response	• Preferred channels/mechanisms for feedback provision • Data sensitivities		✓	✓
Conflict dynamics	Local context and war dynamics	• Overview of the current situation and developments in the local war dynamics	✓	✓	
	Social cohesion	• Nature of relationships between neighbors and other community members • Degree of openness in community culture • Level of equity within the community • Methods of conflict resolution within the community		✓	✓
	Conflict sensitivity	• Cases of community disagreement or tension caused by WFP/other assistance • Involvement of WFP/other organizations' staff in war and political discussions • Instances of WFP/other organizations' assistance being used by local leaders to influence or interfere with their political or economic interests • Likelihood of WFP/other organizations' assistance being used for military advantage • Potential negative community impacts of WFP/other organizations' activities, especially on women and girls		✓	✓

Component	Subcomponent	Indicator	Data collection mode		
			DR	KII	FGD
Socio-cultural and gender norms	Evolving gender roles	Typical roles and responsibilities of women, men, boys, and girls within the community	✓	✓	✓
	Community safety	- Perceived sense of safety - Factors and locations contributing to feelings of insecurity - Coping strategies for addressing feelings of insecurity - WFP/other organizations' actions that enhance/detract from community safety - Types of protection risks affecting women, girls, men, and boys	✓	✓	✓
	Protection risks	Prevalence of harmful practices, discrimination, or violence against specific groups of people based on gender, age, sexual orientation, disability, ethnicity, and other characteristics	✓	✓	✓
	Accountability to affected people	- Awareness of targeted people receiving assistance of the means of obtaining assistance - Preferred methods for accessing information - Preferred methods for communicating with WFP staff - Experience of community involvement in decision-making regarding needed assistance - Optimal ways to ensure community voices are heard		✓	✓
Stakeholder analysis	Identification of relevant stakeholders, and their positions and capacities	Overview of stakeholders, their activities, and the communities they interact with, with a focus on protection and gender	✓	✓	
	Relationships among stakeholders	Nature and quality of relationships between different stakeholders		✓	

Annex B. Desk Review

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PAX. 2025. *Protection of Civilians Ukraine Briefings - Evacuations to Protect Civilians: Ukraine*.

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Tool 7_Qual Interview_Conflict Sensitivity
TOOL2_Menu of questions for Risk Assessment STEP 2
TOOL4_Focus Group Discussions
TOOL6_Stakeholder Analysis
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Annex C. Detailed breakdown of FGDs per oblast

19 in the East

- In **Dnipropetrovsk, Donetsk, Kharkiv, Zaporizhzhia** one FGD was held with each adult women, elderly women, adult men, and elderly men
- In **Dnipropetrovsk** one FGD was held with Roma community representatives, and one with IDPs
- In **Kharkiv** one FGD was held with PWDs

Nine in the South

- In **Odesa** one FGD was held with each adult women and elderly women
- In **Mykolaiv** an FGD was held with each adult men, elderly men, people living with HIV/AIDS, and IDPs
- In **Kherson** one FGD was held with each adult women, elderly women, and Roma community representatives

Six in the North

- In **Sumy** one FGD was held with each adult men and elderly men
- In **Kyiv** one FGD was held with each adult women, elderly women, IDPs, and LGBTQIA+ community representatives

Six in the West

- In **Lviv** one FGD was held with each adult men, elderly men, and PWDs
- In **Vinnitsia** one FGD was held with each adult women, elderly men, and IDPs

Annex D. Anonymized lists of data collection participants

Key Informant Interviews (KIIs)

#	Organization	Date
1	NGO working with Roma communities	08.10.24
2	NGO working with PLHIV/AIDS & LGBTQIA+	08.10.24
3	Local NGO staff	10.10.24
4	NGO working with PLHIV/AIDS	15.10.24
5	NGO working with PLHIV/AIDS & LGBTQIA+	15.10.24
6	Local government representative, Mykolaiv	15.10.24
7	Local government representative, Zaporizhzhia	17.10.24
8	NGO working with IDPs	17.10.24
9	Local government representative, Poltava	21.10.24
10	NGO working with people with disabilities	24.10.24
11	International organization representative	28.10.24
12	International organization representative	16.10.24
13	Cluster representative	24.10.24
14	INGO representative	11.11.24
15	INGO representative	24.10.24
16	Cluster representative	22.10.24
17	Cluster representative	7.11.24
18	International organization representative	30.10.24
19	INGO representative	20.11.24
20	Working group representative	26.11.24

Focus Group Discussions (FGDs)

FGD distribution

#	Oblast	Target Group	Location/Settlement Type of participants	# of participants	Mode	Date
1	Donetsk	Women (18-59)	Respondents invited from different settlement types	6	Remote	20.10.24
2	Donetsk	Men (18-59)	Respondents invited from different settlement types	6	Remote	19.10.24
3	Donetsk	Older women (59+)	Respondents invited from different settlement types	6	Remote	21.10.24
4	Donetsk	Older men (59+)	Respondents invited from different settlement types	6	Remote	22.10.24
5	Kharkiv	Women (18-59)	Bohodukhiv	6	Offli	01.11.24
6	Kharkiv	Men (18-59)	Kamyana Yurga	6	Offli	11.11.24
7	Kharkiv	Older women (59+)	Bohodukhiv	6	Offli	01.11.24
8	Kharkiv	Older men (59+)	Chuguiv	6	Offli	31.10.24
9	Kharkiv	PWD	Chuguiv	6	Offli	31.10.24
10	Dnipropetrovsk	Women (18-59)	Dnipro + participants from Nikopol and Marhanets	6	Offli	23.10.24
11	Dnipropetrovsk	Men (18-59)	Dnipro + participants from Tomakivka	6	Offli	24.10.24
12	Dnipropetrovsk	Older women (59+)	Dnipro + participants from Tomakivka	6	Offli	24.10.24
13	Dnipropetrovsk	Older men (59+)	Dnipro + participants from Marhanets	6	Offli	23.10.24
14	Dnipropetrovsk	Internally displaced people	Dnipro + participants from Nikopol and Kvyi Rih	6 (2 in reserve)	Offli	24.10.24
15	Dnipropetrovsk	Roma community	Marhanets	6	Remote	07.11.24
16	Zaporizhzhia	Women (18-59)	Respondents invited from different settlement types	6	Remote	28.10.24
17	Zaporizhzhia	Men (18-59)	Respondents invited from different settlement types	6	Remote	30.10.24
18	Zaporizhzhia	Older women (59+)	Respondents invited from different settlement types	6	Remote	29.10.24
19	Zaporizhzhia	Older men (59+)	Respondents invited from different settlement types	6	Remote	31.10.24
20	Odesa	Women (18-59)	Odesa city	6	Offli	13.11.24
21	Odesa	Older women (59+)	Ovidiopil	6	Offli	08.11.24

#	Oblast	Target Group	Location/Settlement Type of participants	# of participants	Mode	Date
22	Mykolaiv	Men (18-59)	Respondents invited from different settlement types	6	Remote	26.10.24
23	Mykolaiv	Older men (59+)	Respondents invited from different settlement types	6	Remote	27.10.24
24	Mykolaiv	Internally displaced people	Respondents invited from different settlement types	6	Remote	02.11.24
25	Mykolaiv	HIV/AIDs	Respondents invited from different settlement types	6	Remote	12.11.24
26	Kherson	Women (18-59)	Respondents invited from different settlement types	6	Remote	24.10.24
27	Kherson	Older women (59+)	Respondents invited from different settlement types	6	Remote	25.20.24
28	Kherson	Roma community	Kherson city + oblast	6	Offli	09.11.24
29	Sumy	Men (18-59)	Respondents invited from different settlement types	6	Remote	21.10.24
30	Sumy	Older men (59+)	Respondents invited from different settlement types	6	Remote	21.10.24
31	Kyiv	Women (18-59)	Kyiv + representatives of Kyivska oblast	6	Offli	18.10.24
32	Kyiv	Older women (59+)	Kyiv + representatives of Kyivska oblast	6	Offli	18.10.24
33	Kyiv	Internally displaced people	Makariv	6	Offli	29.10.24
34	Kyiv	LGBTQIA+	Respondents invited from different settlement types	6	Remote	15.11.24
35	Lviv	Men (18-59)	Drohobych	6	Offli	29.10.24
36	Lviv	Older men (59+)	Sosnivka	6	Offli	05.11.24
37	Lviv	People with disabilities	Lviv	6	Offli	09.11.24
38	Vinnytsia	Women (18-59)	Vinnytsia + representatives of Vinnytsia oblast	6	Offli	16.10.24
39	Vinnytsia	Older men (59+)	Vinnytsia + representatives of Vinnytsia oblast	6	Offli	16.10.24
40	Vinnytsia	Internally displaced people	Vinnytsia + representatives of Vinnytsia oblast	6	Offli	16.10.24

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